

**INVESTIGATING CULTURAL FACTORS INFLUENCING SEXUAL AND
REPRODUCTIVE HEALTH RIGHTS (SRHR) SERVICE ACCESS AMONG
ADOLESCENT GIRLS AND YOUNG WOMEN (AGYW), AGED 10-24, IN
NAIROBI, KENYA**

**WG-4: INACCESSIBLE SEXUAL AND REPRODUCTIVE HEALTH RIGHTS
SERVICES FOR ADOLESCENT GIRLS AND YOUNG WOMEN**

ABSTRACT

This study investigates the cultural factors that influence access to sexual and reproductive health rights (SRHR) services among adolescent girls and young women (AGYW) aged 10 to 24 in Nairobi, Kenya. Using a qualitative, feminist standpoint approach, the research team conducted twelve in-depth interviews and one focus group discussion with six additional participants, eighteen AGYW in total, recruited through snowball sampling anchored in Buruburu but extending, through participants' own referral networks, to Embakasi, Kileleshwa, Langata, and Kibera. Findings show that stigma and fear of judgment, rather than the physical unavailability of services, is the central barrier to SRHR access, operating through specific interactions with family members, peers, and healthcare providers. Stigma severity varies by service type: HIV testing carries the most stigma, followed by contraception, then counselling, and its physical and financial consequences vary sharply by socioeconomic context, even though the underlying cultural script of judgment and family silence is broadly similar across the middle-income and informal-settlement communities studied. In the absence of trusted, non-judgmental formal services, AGYW rely heavily on AI tools, social media, and peer networks for SRHR information, despite acknowledging the accuracy risks this carries. The study recommends targeted provider training, community education addressing the misconception that SRHR-seeking implies sexual activity, accuracy safeguards for the digital tools AGYW already use, confidentiality-focused service design in dense settlements, and an expanded scope for future research that includes provider perspectives and younger and male participants.

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CHAPTER ONE

INTRODUCTION

1.1 Background

Authors: Allarassem Bonheur & Collins Bebeth Ifunanyachi

The World Health Organization (WHO) defines young people as individuals aged 10–24, a demographic acutely vulnerable to sexual and reproductive health and rights (SRHR) crises (WHO, 2024). Globally, nearly 12 million girls aged 15–19 are expected to give birth in 2025, and complications from pregnancy and childbirth account for roughly one in every 23 adolescent deaths worldwide (UNICEF, 2025). Women and girls account for an estimated 44% of all new HIV infections globally (Murewanhema et al., 2025), and over 30,000 preventable reproductive deaths occur annually among adolescent girls and young women (AGYW), driven by poor health infrastructure, judgmental provider attitudes, and patriarchal norms that stigmatize contraceptive use (UNFPA, 2024; Guttmacher Institute, 2022; Melesse et al., 2021; WHO, 2023).

The burden of this crisis is not evenly distributed. Sub-Saharan Africa carries a disproportionate share of it: women and girls account for 62% of new HIV infections in the region, compared to 44% globally, and an estimated 3,100 AGYW acquire HIV every week regionwide (Murewanhema et al., 2025). The region's adolescent birth rate is more than double the global average, at over 100 births per 1,000 women in 2021 (Maharaj, 2022). Unequal power dynamics between AGYW and older partners compound these risks, especially where poverty pushes young women toward transactional relationships for survival. Underlying many of these power imbalances are cultural expectations that normalize age-disparate relationships

and discourage young women from negotiating safer sex, a cultural driver that this study examines directly.

Kenya mirrors this regional pattern while revealing its own internal inequalities. Nationally, teenage pregnancy declined from 18% in 2014 to 15% in 2022, yet this conceals disparities ranging from 50% in Samburu County to under 5% in Nyeri County (Kenya National Bureau of Statistics [KNBS], 2023). An estimated 330,000 adolescent girls become pregnant in Kenya every year (UNFPA Kenya, 2025). Youth aged 15 to 24 accounted for 41% of new HIV infections nationally in 2025, while pregnancies among girls aged 10 to 14 rose from 224,333 in 2024 to 235,938 in 2025 (Nation Media Group, 2026). Such wide county-level disparities are difficult to explain through health infrastructure alone, since counties with comparable resource constraints report very different outcomes; cultural variation in norms around early marriage, parental consent, and provider attitudes offers a more plausible explanation, and is the specific gap this study addresses.

Nairobi City County itself records a comparatively low aggregate teenage pregnancy rate of 8.4% (KNBS, 2023), masking sharp inequality within the city. In informal settlements such as Mathare and Kibera, AGYW face compounded poverty and limited health access; the median age of sexual debut among those surveyed was 16 years, with only 26% reporting condom use at last sex (Ziraba et al., 2018). The 2025 closure of DREAMS has withdrawn HIV outreach for roughly two million AGYW regionwide (UNAIDS, 2025).

1.2 Problem Statement

Authors: John Batani, Allarassem Bonheur & Alungat Patience

Despite SRHR being recognized as a fundamental human right (Orlando et al., 2025; Sedgh et al., 2025), AGYW continues to face suboptimal access to SRHR services. Existing research

attributes this largely to provider attitudes, structural barriers, and declining donor funding (Dzinamarira et al., 2026; Melesse et al., 2021). AGYW in Southern and Eastern Africa remain twice as likely to be living with HIV as their male counterparts, and barriers to access continue to drive unintended pregnancies, school dropout, and economic marginalization (Ayalew et al., 2022; UNFPA Rwanda, 2024). Much of this research has been conducted at national or regional level, drawing on health system data and large scale surveys, which makes it useful for identifying broad patterns but less able to explain why access remains so uneven between communities facing similar economic conditions (Ninsiima et al., 2021).

Much of the existing literature on SRHR access among AGYW in Sub-Saharan Africa, however, examines structural, economic, and health system barriers in general terms, without specifically interrogating the cultural factors that shape how AGYW perceive and pursue SRHR information and services (Ninsiima et al., 2021; Ouahid et al., 2025). Yet culture is a key factor that influences behaviour, including attitudes to accessing SRHR, and how it interacts with these structural barriers is not yet well understood, particularly within urban informal settlements, where AGYW face some of the region's highest rates of HIV infection, unintended pregnancy, and maternal mortality (Okumu et al., 2023; Chola et al., 2023). This gap has become more urgent following the loss of major donor funded programmes such as DREAMS, which has left a critical vacuum in how AGYW access reproductive health information and support, and about which little is currently known. It is also unclear what social or physical consequences AGYW themselves associate with seeking this information within their own families and communities, a dimension that is easily overlooked in studies that focus only on health system barriers. While cultural factors can in principle both enable and constrain SRHR access, this study focuses specifically on the barriers AGYW themselves identify, since these remain the least documented dimension in the literature reviewed above. This study, therefore, seeks to fill that gap by investigating the cultural barriers to SRHR access among AGYW aged

10 to 24 in Nairobi, Kenya, including how these barriers interact with one another and shape AGYW's everyday health-seeking decisions.

1.3 Research Questions and Objectives of the Study

Authors: Lvy Oguye, Abubakar Diallo & Joy Chepkorir Bett

1.3.1 Objectives of the Study

General Objective

To investigate the cultural factors that influence access to sexual and reproductive health and rights (SRHR) services among adolescent girls and young women (AGYW) aged 10–24 in Nairobi, Kenya.

Specific Objectives

- To identify the cultural barriers that prevent AGYW aged 10–24 from accessing SRHR services in Nairobi, Kenya.
- To explore AGYW's perceptions of how cultural biases and moral policing by healthcare providers within clinics shape the youth-friendliness and accessibility of SRHR services in Nairobi, Kenya.
- To identify the social, familial, and physical risks that AGYW aged 10–24 associate with being discovered seeking SRHR services in Nairobi, Kenya.

1.3.2 Research Questions

RQ1: What cultural barriers prevent AGYW from accessing SRHR services in Nairobi, Kenya?

RQ2: In what ways do clinical environments and the personal biases of healthcare workers create a culturally unwelcoming environment for AGYW seeking SRHR services in Nairobi, Kenya?

RQ3: What social, familial, or physical risks do AGYW face within the community if they are discovered seeking information about SRHR services in Nairobi, Kenya?

1.4 Relevance of the Study

Authors: Lvy Oguye & Abubakar Diallo

Cultural factors, most notably patriarchal power dynamics that enable reproductive coercion within intimate partnerships, shape AGYW's decisions to seek SRHR services, contributing to unintended pregnancies, STIs, and unmet contraceptive needs (Ninsiima et al., 2021). With Sub-Saharan Africa hosting one of the world's largest youth populations, understanding these dynamics is critical for designing responsive, youth-friendly interventions. This study equips policymakers, healthcare providers, and educators with culturally sensitive strategies, aligning with SDGs 3 and 5 and the AU's Agenda 2063 (United Nations, 2015; African Union Commission, 2015). Concretely, programmes that fail to account for these cultural dynamics risk repeating the access gaps documented in Section 1.1 even where funding and facilities are adequate, which makes cultural insight a precondition for effective SRHR programming rather than an optional add-on.

1.5 Significance of the Study

Authors: Alungat Patience, Denis Njoroge & Joy Chepkorir

Employing feminist standpoint theory, this study positions AGYW as experts on their own circumstances, generating qualitative insights into how cultural barriers are reproduced at community, family, and institutional levels, insights quantitative analyses cannot capture. This study's focus on Nairobi addresses a documented evidence gap: context-specific data from East Africa, a sub-region where AGYW face the world's highest rates of HIV infection, unintended pregnancy, and maternal mortality, remain scarce (Ouahid et al., 2025). Evidence confirms that cultural factors and intersecting stigmas shape how AGYW evaluate seeking care (Okumu et al., 2023; Chola et al., 2023). By centering AGYW's own accounts of how these cultural barriers operate, this study offers policymakers, healthcare providers, and researchers an evidence base for designing gender-transformative SRHR programmes and policies, grounded in feminist standpoint theory's commitment to treating marginalized groups as authoritative sources of knowledge about their own lives (see 1.4 for the corresponding policy alignment). In doing so, the study moves the conversation beyond documenting that access gaps exist toward explaining, in AGYW's own terms, why they persist, which is the level of detail programme designers need in order to act.

CHAPTER TWO

LITERATURE REVIEW

Introduction

This chapter presents a combined literature review addressing all three research questions guiding this study: the cultural barriers that prevent AGYW from accessing SRHR services in Nairobi, Kenya; the ways in which clinical environments and the personal biases of healthcare workers create a culturally unwelcoming environment for AGYW seeking these services; and the social, familial, and physical risks AGYW associate with being discovered seeking SRHR information or services. The review is organized around three themes. The first, the vicious cycle of reproductive coercion and socioeconomic marginalization, treats patriarchal control over reproductive decisions and economic dependency as mutually reinforcing forces that restrict AGYW's autonomy in Nairobi. The second, clinical environments and healthcare worker biases as barriers to culturally responsive SRHR services, examines how provider attitudes and facility conditions extend these same cultural dynamics into the clinic itself. The third, discovery and its consequences, draws on stigma research from Ghana, Zambia, and Kenya to examine what is known about the social, familial, and physical fallout AGYW face once their SRHR-seeking becomes known to others. Read together, the three themes show that the barriers AGYW encounter in Nairobi do not stop at the household or the community; they persist into the health system that is meant to serve them, and they continue to shadow AGYW after a service has been sought, in the form of anticipated or realized consequences at home. What the existing literature does not yet capture is a Nairobi-specific account that spans both a middle-income estate and an informal settlement within a single study, or one built from AGYW's own anticipatory accounts of what they expect would happen to them personally,

rather than from retrospective reports, secondhand accounts, or the perspectives of health workers and researchers. This is the specific gap the fieldwork that follows this review is positioned to close.

THE “VICIOUS CYCLE” OF REPRODUCTIVE COERCION AND SOCIOECONOMIC MARGINALIZATION (THE INTERSECTION)

Authors: Joy Chepkorir Bett, Abubakar Diallo, Lvy Oguye

Reproductive coercion (RC) is a form of gender-based violence that undermines the reproductive autonomy of individuals capable of becoming pregnant through behaviors such as contraceptive sabotage, pregnancy pressure, and interference with clinical access (Miller et al., 2010; Lévesque et al., 2025). In Nairobi, these behaviors do not occur in a vacuum. They are underwritten by cultural expectations that assign reproductive decision-making to male partners, parents, or community elders rather than to AGYW themselves, and by religious and social norms that treat premarital sexuality and contraceptive use among unmarried young women as improper (Kinaro et al., 2019; Langat et al., 2024). Lévesque et al. (2025) argue that RC is best understood across interpersonal, community, and societal levels rather than as an isolated act between partners, since providers who restrict contraceptive counselling and social norms that place the burden of fertility control on women both extend RC beyond the household and into the institutions AGYW rely on for care. This ecological view is borne out in Nairobi's own data: nearly one in five partnered AGYW report lifetime RC, and 93.2% report transactional elements within their relationships that further complicate their ability to negotiate contraceptive use (Decker et al., 2021). Cultural stigma, in other words, does not simply discourage AGYW from seeking SRHR services. It actively shapes the terms on which their partners, families, and communities permit them to do so.

Socioeconomic marginalization intensifies these cultural dynamics rather than operating alongside them. Poverty and economic dependence push young women in Nairobi's informal settlements toward transactional relationships in which negotiating condom use or contraceptive access carries a direct financial cost, deepening the same power imbalance that enables coercion (Ninsiima et al., 2021; Omojemite, 2026). Regional evidence helps explain why: Dozier et al. (2025) found that women with more than a secondary education had substantially lower odds of experiencing RC than women with no formal schooling, identifying education as a significant protective structural factor, while Omojemite (2026) centers financial dependence and unemployment as the dominant predictors of what the study terms forced motherhood, arguing that economic dependency normalizes male authority over reproductive decisions. Raihen et al. (2023) reinforce this pattern, showing that patriarchal power within intimate partnerships most constrains the choices of women who are least able to exit the relationship economically. For AGYW in Nairobi's informal settlements, where school dropout and limited income opportunities are already common, this relationship compounds an existing vulnerability rather than introducing a new one.

This produces a feedback loop rather than a single point of harm. Boyce et al. (2020) and Wood et al. (2020), drawing on qualitative work with women and girls in Nairobi's family planning clinics, describe partners who pressure AGYW into pregnancy or deliberately damage their contraceptive supplies, prompting many women to conceal contraceptive use altogether as a short-term safety strategy. In Nairobi specifically, 15.6% of partnered AGYW report non-consensual condom removal, and discovery of concealed contraceptive use frequently triggers partner violence and the destruction of clinical records, which blocks future access to SRHR services entirely (Decker et al., 2021). The unintended pregnancies that follow this cycle routinely end schooling or vocational training, deepening the same financial dependence that made the original coercion possible, so that socioeconomic marginalization and reproductive

coercion in Nairobi reinforce one another over time rather than resolving after a single incident (Ayalew et al., 2022).

Some evidence suggests this cycle can be interrupted, at least at the clinical level. A cluster-controlled trial of the ARCHES intervention across six private Nairobi clinics found that training providers to offer private, partner-separated counselling, including contraceptive methods that are harder for a partner to detect, produced a significantly greater reduction in attitudes justifying reproductive coercion relative to standard care by six-month follow-up (Uysal et al., 2023). Community health worker training piloted in Malawi offers a comparable model, shifting CHWs from judgmental gatekeepers into non-judgmental mentors who support adolescent mothers' social and educational reintegration (African Population and Health Research Center & Centre for Social Research, University of Malawi, 2021; Otukpa et al., 2025). Because this model was developed for adolescent mothers outside Kenya, however, it remains unclear whether it would address the specific clinical access barriers documented among AGYW in Nairobi's informal settlements, a question the ARCHES trial's Nairobi-based results only partially answer since the two interventions test different points in the same cycle.

CLINICAL ENVIRONMENTS AND HEALTHCARE WORKER BIASES AS BARRIERS TO CULTURALLY RESPONSIVE SEXUAL AND REPRODUCTIVE HEALTH SERVICES FOR ADOLESCENT GIRLS AND YOUNG WOMEN

Authors: Collins Bebeth , Denis Njoroge

Even where AGYW in Nairobi overcome the cultural and economic barriers described above, the health facilities they eventually reach frequently reproduce the same judgment they anticipated from their communities. Provider bias occurs when healthcare workers allow personal beliefs, values, or assumptions to shape the services they provide instead of relying solely on professional standards (Solo & Festin, 2019). In Kenya, Godia et al. (2013) found that

some healthcare providers viewed adolescent sexual activity as a moral issue rather than a health issue, which made it difficult for providers serving Nairobi's youth to separate personal belief from professional responsibility. O'Malley et al. (2021) reported a similar pattern among providers serving AGYW, who expressed concern that offering contraceptives or HIV prevention services could be seen as encouraging sexual behavior. Taken together, this evidence indicates that providers in Nairobi's clinics often act as gatekeepers, determining adolescents' access to services based on perceived social acceptability rather than clinical need, a pattern that closely mirrors the community-level gatekeeping already described in the discussion of reproductive coercion above.

Beyond individual attitudes, the physical and social environment of Nairobi's health facilities shapes whether AGYW feel safe enough to attend at all. Corley et al. (2022) identify privacy, confidentiality, and respectful communication as essential components of youth-friendly SRHR services, noting that young women may avoid services altogether if they fear being recognized or exposed to neighbors or relatives, a risk that is heightened in the dense, closely networked informal settlements where much of Nairobi's AGYW population lives. Kenyan evidence from Godia et al. (2014) found that adolescents associated clinics with long queues, inadequate consultation spaces, overcrowded facilities, and uncomfortable interactions with staff, with some participants reporting that they felt watched or embarrassed during consultations. These experiences reduce trust and discourage future service use, an effect that compounding facility conditions and negative staff attitudes make more severe still (Chakupewa & Mmari, 2021). A significant limitation runs through this evidence, however: most of it draws from girls who already attended a Nairobi facility, while girls who never attended because they anticipated stigma or judgment are largely absent from the data, meaning the true scale of avoidance in the city may be considerably larger than current research shows.

Unwelcoming experiences in Nairobi's clinics are not solely the product of individual providers. They are also shaped by the broader health system within which those providers work. Sidamo et al. (2023) identify poor provider training, weak adolescent-focused systems, and institutional practices as barriers to youth-friendly care, while Chakupewa and Mmari (2021) argue that negative staff attitudes combined with poor facility conditions jointly discourage adolescents from seeking care. There is evidence that these problems are addressable within Kenya specifically: Mutea et al. (2023) found that combining provider training with practical changes, such as improved clinic organization and extended service hours, increased adolescents' comfort with SRHR services. Still, most intervention studies, including this one, measure whether service use increased in the short term rather than whether provider attitudes changed durably, and little evidence exists on whether improvements in Nairobi's clinics persist once external support ends.

Recent literature increasingly frames provider bias in Nairobi as a reproductive rights issue rather than a service delivery issue alone. Senderowicz (2020) introduces the concept of contraceptive autonomy, the principle that individuals should be able to make reproductive decisions freely and without pressure, positioning healthcare workers as actors who can either support or restrict young women's choices. Tavrow (2026), writing on government health facilities in rural Kenya, extends this by describing healthcare workers as gatekeepers whose attitudes determine whether young people feel accepted and are willing to return for future services, a dynamic that plausibly holds with even greater force in Nairobi's higher-volume public facilities. Provider bias, in this framing, is not an isolated professional failing but an extension of the same cultural and religious norms that stigmatize adolescent sexuality in the wider Nairobi community (Sidamo et al., 2023), which is precisely why the vicious cycle described in the first theme of this review does not end at the household. It continues, largely unbroken, into the clinic.

DISCOVERY AND ITS CONSEQUENCES: SOCIAL, FAMILIAL, AND PHYSICAL RISK IN SEEKING SEXUAL AND REPRODUCTIVE HEALTH SERVICES AMONG ADOLESCENT GIRLS AND YOUNG WOMEN

Authors: Joy Chepkorir Bett

Across Sub-Saharan Africa, stigma researchers have converged on a similar finding: AGYW who are discovered, or who fear being discovered, seeking sexual or reproductive health services face consequences that extend well beyond the clinic visit itself. Hall et al. (2018), drawing on interviews with young women in Ghana, describe SRH stigma as operating across five interlocking domains: community norms that label sexually active adolescents as bad girls, enacted stigma in the form of gossip and mistreatment, internalized shame, non-disclosure and secret-keeping as a coping strategy, and, less commonly, resilience through social support. Strikingly, adolescents in a very different part of the region use almost identical language. Harrington et al. (2021), interviewing adolescent girls in Kenya's Nyanza region, found that participants used a single word, spoiled, a direct translation from Luo, to describe girls who were sexually experienced, futures ruined by early pregnancy, and fertility participants believed could be damaged by contraceptive use, collapsing three distinct sources of stigma into one label. That two unrelated studies, conducted in different countries and languages, independently arrive at such closely related labeling logics suggests this is not a culturally idiosyncratic finding but a recurring feature of how communities in the region regulate adolescent sexuality.

Much of this stigma is enacted rather than merely anticipated, and clinics themselves are frequently the source of unwanted disclosure rather than a refuge from it. Meek et al. (2022), interviewing health workers in Zambia, found that gossip among staff and the layout of shared waiting rooms routinely exposed AGYW attending family planning or HIV services to

community members who recognized them, with health workers openly acknowledging that colleagues talk among themselves about which young women they have seen and why. This matches Corley et al.'s (2022) identification of privacy and confidentiality as essential, frequently absent, components of youth-friendly care, and Sidamo et al.'s (2023) finding that fear of being seen at a clinic by a family member, neighbor, or teacher is among the most commonly reported barriers to adolescent SRH service use across the region. Family reactions to discovery, where documented, span a similarly wide range: Hall et al. (2018) describe adolescents being rejected by family, dismissed from school, and shut out of their churches, while Meek et al. (2022) describe AGYW living with HIV facing physical isolation within their own homes, imposed by family members who feared contracting the virus themselves. Neither study, however, was conducted in Nairobi, and neither compares a middle-income setting against an informal settlement within the same sample, leaving open whether family reactions of this kind vary by socioeconomic context in the way Nairobi-specific research on reproductive coercion (Boyce et al., 2020; Decker et al., 2021) suggests provider and partner responses do.

The physical stakes of discovery, and of the secrecy adopted to avoid it, are the least documented dimension of this literature but the most severe where they do appear. Harrington et al. (2021) recount two Kenyan participants' accounts of this dilemma. One IDI participant described a thirteen-year-old friend who wanted an abortion after becoming pregnant, was warned it could kill her, grew suicidal as the pregnancy continued, and eventually gave birth. Another, a sixteen-year-old, described weighing an unsafe abortion against her fear that her father might beat her to death if she carried the pregnancy to term, concluding that she might not survive either path. Hall et al. (2018) document a comparable pattern in Ghana, where adolescents described verbal, psychological, and physical mistreatment, including being beaten, following disclosure of sexual activity or pregnancy. This connects directly to the

concealment behavior described in the discussion of reproductive coercion above: Boyce et al. (2020) and Decker et al. (2021) found that AGYW in Nairobi who conceal contraceptive use from a partner to avoid conflict face a comparable trade-off, accepting one risk to avoid another. What the evidence reviewed here adds is that this trade-off is not confined to intimate partnerships; it operates at the level of family and community as well, and it can be severe enough that participants describe it in the same breath as suicide and unsafe abortion, rather than as a lesser social inconvenience.

Taken together, this literature establishes that discovery, or the anticipation of it, carries real social, familial, and in some cases physical consequences for AGYW seeking SRHR services, and that the underlying stigma logic, labeling, gossip, family rejection, and secrecy as a survival strategy, recurs with striking consistency from Ghana to Zambia to rural Kenya. What it does not yet provide is an account built specifically from Nairobi, spanning both a middle-income estate and an informal settlement within the same study, or one grounded in what AGYW themselves anticipate would happen to them personally rather than in retrospective, secondhand, or health-worker accounts. Addressing that gap directly is the specific contribution of the third research question guiding this study.

Research Gaps

Several gaps recur across the literature reviewed in this chapter. First, the majority of quantitative studies on reproductive coercion in Nairobi, including Decker et al. (2021) and Wood et al. (2023), rely on cross-sectional designs, which establish that socioeconomic marginalization and RC co-occur but cannot confirm which comes first or whether the cycle intensifies the longer it continues. Longitudinal research tracking AGYW through this cycle over time is largely absent from Nairobi-specific literature.

Second, existing evidence on both reproductive coercion and clinical bias is concentrated among AGYW who already attend clinics or who are already partnered, leaving two related populations understudied: AGYW who avoid care altogether because they anticipate stigma or judgment, and unmarried or out-of-school AGYW whose experiences may differ from the clinic-attending samples most studies draw on (Ninsiima et al., 2021; Godia et al., 2014).

Third, the literature overwhelmingly centers on women's own accounts and, within clinical settings, providers' accounts, examined largely as separate bodies of evidence. Raihen et al. (2023) and Omojemite (2026) note the near total absence of research into male partners' or parents' motivations, community norms that enable coercion, or male engagement as a prevention strategy, while Godia et al. (2013, 2014) study providers and adolescents as separate populations rather than examining how the two groups negotiate cultural expectations together during an actual clinical encounter.

Fourth, while Lévesque et al. (2025) provide a useful ecological framework for understanding RC, and Mutea et al. (2023) demonstrate that combined provider training and structural change can improve service use in Kenya in the short term, neither line of research has been extended into evidence on whether these gains persist once external support ends, or whether the ecological framework itself has been empirically validated for Nairobi's specific institutional and cultural structures.

Finally, and most directly relevant to this study, the literature reviewed in the third theme above establishes that discovery-related stigma is a real and often severe phenomenon across the region, but it does so from Ghana, Zambia, and rural Kenya rather than from Nairobi specifically, and none of it compares a middle-income estate against an informal settlement within a single sample, the specific comparison this study's realized sampling, described in Chapter Three, makes possible. Nor does it ask AGYW to speak in their own anticipatory

terms, what they themselves expect would happen, rather than reporting what has already happened to them or to someone they know. This is precisely the third research question this study is designed to answer, and it is the gap that the qualitative, descriptive phenomenological fieldwork described in Chapter Three is positioned to close.

Conclusion

The literature reviewed in this chapter shows that the cultural barriers preventing AGYW from accessing SRHR services in Nairobi operate as a single connected system rather than as separate obstacles. Reproductive coercion and socioeconomic marginalization reinforce one another at the level of the household and the community, while provider bias and unwelcoming clinical environments extend the same cultural norms into the health facilities AGYW eventually reach, and stigma tied to discovery, documented elsewhere in the region though not yet in Nairobi itself, shadows AGYW both before and after they seek care. None of the three themes, on its own, resolves the barriers AGYW face; a woman who negotiates her way past a coercive partner may still meet a judgmental provider on the same visit, a clinic that trains its providers well cannot, on its own, address the economic dependency that makes coercion possible at home, and even a successful, private clinic visit does not remove the risk that a neighbor, relative, or classmate will learn of it afterward. What the evidence does not yet capture, in Nairobi specifically, is longitudinal data on how the reproductive coercion cycle develops over time, the experiences of AGYW who avoid care entirely, a comparison of discovery-related risk across a middle-income estate and an informal settlement within the same study, and, most importantly for this study, what AGYW themselves anticipate would happen within their families and communities if their efforts to seek SRHR services were discovered. Addressing this last gap through AGYW's own accounts, gathered directly from

across Nairobi, is the specific contribution the fieldwork that follows this review is designed to make.

CHAPTER THREE

METHODOLOGY

3.1 Introduction to the Methodology

This section presents the methodology adopted for this study. It outlines the research approach and design, the study area and population, the sample and the procedure used to select participants, the methods and tools used to collect data, and the steps taken to manage, analyze, and safeguard the quality of the data. It closes with a discussion of the ethical considerations relevant to working with adolescent girls and young women (AGYW) on a sensitive topic such as sexual and reproductive health and rights (SRHR). In keeping with the qualitative, experience-centred orientation of this study (see Significance of the Study), a qualitative approach using a descriptive phenomenological design is adopted, with the aim of describing and understanding the cultural barriers, sources of support, and risks that AGYW associate with accessing SRHR services in Nairobi, Kenya.

3.2 Research Design

3.2.1 Research Approach

This study adopts a qualitative research approach. Qualitative research is concerned with understanding the meaning people attach to their experiences and with exploring a phenomenon in depth, rather than measuring it numerically (Creswell & Creswell, 2018). This approach is appropriate for the present study because its guiding objectives and research questions are framed around “what” questions, namely what cultural barriers AGYW face, what sources they rely on, and what risks they associate with seeking SRHR services. These

questions seek to explore meaning and lived experience rather than to measure frequency or test a hypothesis, which places the study firmly within the qualitative tradition. This choice is also consistent with the feminist standpoint lens adopted by the study, which treats AGYW as experts on their own circumstances and prioritizes their own accounts over externally imposed measures (see Significance of the Study).

3.2.2 Research Design

The study adopts a descriptive phenomenological design. Descriptive research is designed to provide an accurate account of the characteristics of a particular situation, population, or phenomenon as it currently exists (Kothari, 2004). A descriptive design is appropriate here because the study seeks to describe, rather than to causally explain or statistically compare, the cultural barriers, support sources, and risks that AGYW in Nairobi encounter. Phenomenological research, in turn, is used when the researcher seeks to understand the lived experiences of people navigating a particular phenomenon, often one that is sensitive or stigmatized in nature (Creswell, 2013). This study draws on phenomenology because it is not enough to list barriers and risks; it also seeks to understand the meaning AGYW themselves attach to these experiences. Combining the two, the study is positioned as descriptive phenomenological: it describes the situation of AGYW accessing SRHR services in Nairobi while also capturing the deeper meaning participants attach to those experiences.

3.3 Study Area

This study was originally planned as a single-site study in Buruburu, a middle-income residential estate within Nairobi County, Kenya. Existing SRHR research on AGYW in Nairobi has concentrated heavily on informal settlements, where compounded poverty and documented vulnerability to early sexual debut and inconsistent contraceptive use have attracted sustained

scholarly attention (Ziraba et al., 2018). Beguy et al. (2009), drawing on comparative data from slum and non-slum settlements in Nairobi, similarly observe that adolescent sexual behaviour research has focused disproportionately on informal settlements, leaving non-slum urban communities comparatively understudied. Buruburu was therefore selected as an appropriate entry point precisely because it falls outside this concentration of existing evidence. Practically, the study planned to reach AGYW in Buruburu through two community entry points: adolescent participants identified with parental awareness through estate-based family and youth networks, and older AGYW reached through a salon in Buruburu frequented by young women in the target age range. These same entry points form the seed points for the two snowball sampling chains described in Section 3.6. In practice, because recruitment proceeded through participants' own peer, family, and community networks rather than along fixed geographic boundaries, the realized sample was not confined to Buruburu. The twelve AGYW who ultimately took part in individual interviews were living in five different parts of Nairobi at the time of data collection: Buruburu, Embakasi, Kileleshwa, and Langata, middle-to-upper income residential areas broadly comparable in profile to the original target estate, and Kibera, one of Nairobi's largest informal settlements, reached through a separate community contact. Rather than treating this spread as a deviation from the study's aims, this study treats it as an opportunity: it allows the comparison the study originally set out to make, between AGYW experiences in a middle-income estate and those documented in the informal-settlement literature, to be examined directly within a single dataset. This comparison is developed in Chapter Four.

3.4 Study Population

The study population is AGYW aged 10–24 years residing in Nairobi. Recruitment was anchored in Buruburu but, as described in Section 3.3, extended through participants' own

referral networks to Embakasi, Kileleshwa, Langata, and Kibera as well. This population includes both in-school and out-of-school AGYW, since cultural barriers to SRHR access often differ between the two groups: in-school AGYW may encounter school-based gatekeeping or curriculum-related gaps, while out-of-school AGYW may face barriers rooted more in family expectations or economic dependency. Including both groups allows the study to capture a fuller range of cultural barriers, support sources, and risks than a narrower population would. This population is appropriate for the study because its objectives are specifically concerned with AGYW as a group, and because the age range of 10 to 24 reflects the WHO definition of young people used throughout this research (WHO, 2024, as cited in the Background). Given the study's five-day data collection timeline, the study population is limited to AGYW directly; key informant perspectives from healthcare providers and community gatekeepers are not included in this round of data collection, and Objective 2 of this study is addressed instead through AGYW's own accounts of their interactions with clinical environments and healthcare providers, consistent with the study's overall focus on AGYW as experts on their own experience.

3.5 Study Sample

The study will recruit a total of 18 participants, drawn from the study population described above. This includes 12 participants for individual in-depth interviews and 6 participants for one focus group discussion, both with AGYW. In-depth interviews are expected to last approximately 45 to 60 minutes each, and the focus group discussion approximately 60 to 90 minutes. There is no overlap between the two groups.

Method	Number of Participants
In-depth interviews(IDIs) with AGYW	12
Focus group discussion (FGD) with AGYW	6

Total	18
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This sample size is appropriate for a qualitative study of this kind, where the goal is depth of understanding rather than statistical generalizability. Twelve in-depth interviews is within the range recognized as sufficient to reach thematic saturation in qualitative research of this kind (Guest et al., 2006), offering a reasonable depth of insight into individual experiences of cultural barriers and risk, while a single focus group of six participants is included to capture shared community-level perceptions and norms that are more readily surfaced in a group setting than in one-on-one interviews. The split also reflects the sensitivity of the topic: individual interviews allow participants to disclose personal experiences of stigma or risk without the presence of peers, while the focus group is reserved for questions where shared, less personally identifying community norms are the focus. Given the study's five-day data collection timeline, key informant interviews with healthcare providers and community gatekeepers are not included in this round of data collection; Objective 2 is instead addressed entirely through AGYW's own accounts of their experiences with clinical environments and healthcare providers, consistent with the AGYW-centred, feminist standpoint orientation of the study.

3.6 Sampling Procedure

AGYW participants will be selected using snowball sampling, a non-random procedure in which initial participants, identified through a community gatekeeper, assist in identifying further participants who meet the study's criteria. Snowball sampling is widely recommended for reaching populations who are difficult to identify through formal records and for studying sensitive topics where trust between the researcher and participant is essential (Naderifar et al., 2017). It is appropriate for this study for two reasons. First, AGYW who have direct experience of the cultural barriers and risks under investigation are not easily identified through formal

lists or institutional records, particularly given the sensitivity of SRHR-related experiences; existing peer and trust networks are a more realistic route to reaching them. Second, because the topic involves disclosure of stigmatized or sensitive experiences, participants are more likely to take part, and to speak candidly, when approached through a trusted referral from someone they already know, rather than through a cold approach from an unfamiliar researcher. The study drew on separate seed-and-snowball chains to reach AGYW across the target age range, anchored in Buruburu but not confined to it. One chain begins with adolescent participants identified through estate-based family and youth networks, with initial contact made with parents' or guardians' awareness from the outset; these seed participants then assist in identifying peers who meet the study's criteria. Another chain begins with older AGYW reached through a salon in Buruburu frequented by young women in the target age range, whose regular clients and their peers form the basis for further referrals. A further chain, reached through a separate community contact in Kibera, extended the sample to an informal settlement. Because referrals followed participants' own friendship and family networks rather than fixed geographic boundaries, these chains ultimately reached AGYW in Embakasi, Kileleshwa, and Langata as well. Using multiple chains, rather than one, is intended to reduce over-reliance on a single referral network and to help capture in-school, parent-connected adolescents, older or out-of-school young women, and AGYW living in both middle-income and informal-settlement contexts. The research team monitored the diversity of the resulting sample, for example across in-school and out-of-school status, area of residence, and referral chain, to avoid over-relying on any single referral pathway. Recruitment continued until the target of 12 IDI participants and 6 FGD participants was reached.

3.7 Data Collection Methods and Tools

Data will be collected using a combination of individual in-depth interviews (IDIs) and one focus group discussion (FGD), both qualitative methods consistent with the study's overall approach.

3.7.1 In-Depth Interviews and Focus Group Discussion(AGYW)

In-depth interviews will be conducted with 12 AGYW participants using a semi-structured interview guide, and are expected to last approximately 45 to 60 minutes each. This format allows the interviewer to ask a consistent set of core questions across all participants while leaving room to follow up on points specific to each participant's experience, which is important given the personal nature of the topic. The guide is organized into thematic sections corresponding to the study's research questions, beginning with a biographical section before moving into the substantive themes, so that questions are grouped by topic rather than presented as a single undifferentiated list. A sample of the interview guide is included as Appendix A.

The focus group discussion will be conducted with 6 AGYW participants using a semi-structured FGD guide, organized into the same thematic sections as the IDI guide and following the same core questions framed for group discussion, with the addition of a probe on whether participants have heard of experiences differing for AGYW in other communities. This line of inquiry was raised during supervision but is retained at the level of an informal probe rather than a formal research objective, consistent with the qualitative scope of this study. The FGD is expected to last approximately 60 to 90 minutes. The FGD guide is included as Appendix B.

Both the IDIs and the FGD will be conducted primarily in Kiswahili and Sheng, with English used where participants are more comfortable in English, reflecting the everyday language use of the study population across these Nairobi communities. A bilingual member of the research team will conduct the sessions and will be responsible for translating and transcribing the

recordings into English for analysis, with the original-language version retained alongside the translation so that meaning is not lost in the process.

All sessions across both methods will be audio recorded with participants' consent, in addition to handwritten notes taken by a note-taker present at the focus group discussion.

3.8 Data Management and Analysis

3.8.1 Data Management

Audio recordings from the interviews and focus group discussion will be transferred from the recording device to a password-protected laptop and a password-protected cloud storage folder as soon as possible after each session, after which the original recording on the device will be deleted. Each participant will be assigned a unique code (for example, P1 for the first AGYW interview participant) rather than being identified by name in any transcript, field note, or analysis document. Only members of the research team will have access to the raw recordings and transcripts; any data shared with supervisors or included in the final report will be anonymized.

3.8.2 Data Analysis

Data will be analyzed using thematic analysis, following Braun and Clarke's (2006) six-phase framework: familiarization with the data, generation of initial codes, searching for themes, reviewing themes, defining and naming themes, and producing the final write-up. This approach is appropriate for the study because it allows patterns and themes to be identified across the IDI and FGD transcripts in relation to the study's objectives, namely cultural barriers, sources of support, and risks associated with seeking SRHR services, while remaining grounded in participants' own words and meanings rather than imposing a predetermined coding framework.

3.9 Data Quality Control

To ensure the trustworthiness of the findings, this study addresses the four quality criteria proposed by Lincoln and Guba (1985) for qualitative research:

- **Credibility:** After analysis, a summary of the emerging themes will be shared with a small number of participants who agreed to follow-up contact, to confirm that the themes reflect their actual experiences and perspectives (member checking). Data triangulation is also achieved across the two data sources collected, individual in-depth interviews and the focus group discussion, allowing individually disclosed experiences to be considered alongside shared, community-level perceptions of the same barriers.
- **Transferability:** The study area, population, and sampling procedure are described in sufficient detail in this section to allow readers to judge whether the findings might apply to AGYW in other middle-income urban communities with similar characteristics.
- **Dependability:** The research process, including the IDI and FGD guides, sampling procedure, and analysis steps, is documented clearly in this section and will be retained as a record so that another researcher could follow the same process.
- **Confirmability:** The research team will keep a brief reflexive journal during data collection and analysis, noting any personal assumptions or reactions, to help ensure that the themes that emerge are grounded in participants' data rather than in the researchers' own expectations.

3.10 Research Ethics

This study involves AGYW, including minors, discussing a sensitive topic, and therefore requires careful attention to ethical conduct throughout, in line with WHO (2023) guidance on

engaging young people in health-related research. Given the inclusion of participants as young as 10 years, the research team will apply heightened safeguarding measures for participants under 16, including the presence of a trusted community adult during sessions and age-appropriate question framing. The IDI guide will be adapted to avoid questions that require participants to self-disclose specific personal experiences of abuse or risk.

Informed consent: All AGYW participants will be informed, in a language they understand, about the purpose of the study, what participation involves, the voluntary nature of participation, and their right to skip any question or stop at any time. Consent and assent are documented using separate, purpose-specific forms rather than embedded in this document: participants aged 18 to 24 will complete an adult consent form in their own right, while for participants under 18, written or verbal consent will be sought from a parent or guardian using a separate guardian consent form, in addition to the participant's own assent recorded on a separate, age-appropriate assent form, so that no minor is enrolled without both layers of agreement. Consent and assent will be sought in a private setting to reduce pressure to agree in front of others.

Confidentiality and anonymity: Participants will not be identified by name in any transcript, recording label, or written report; each will be assigned a code. Recordings and transcripts will be stored securely as described in the Data Management section above, and any quotations used in the final report will be anonymized and checked to ensure they cannot identify the speaker.

Right to withdraw: Participants will be informed, both at the start and during the session, that they may withdraw from the study at any point, including after the interview or focus group has taken place, without any negative consequence, and without needing to give a reason.

Data protection: All recordings, notes, and transcripts will be stored on password-protected devices and cloud storage accessible only to the research team, and will be deleted from recording devices once securely transferred, as described above.

Do no harm: Given the sensitivity of the topic and the possibility that some questions may bring up distressing experiences, sessions will be conducted by trained members of the research team using a calm, non-judgmental approach, and will be paused or stopped if a participant becomes distressed. The research team will identify and share the contact details of a local psychosocial support or counselling service with all AGYW participants, regardless of whether they show signs of distress, so that support is available if needed after the session.

Institutional approval: No formal institutional ethics clearance has been sought for this study at this stage, as this is a bootcamp training exercise rather than a funded or published study. In place of formal clearance, the research team will seek explicit permission from the community gatekeepers, including parents or guardians and the salon owner facilitating access to participants, and will follow the consent and protection measures outlined above as the primary safeguard for participants.

CHAPTER FOUR

FINDINGS

4.1 Introduction to Findings

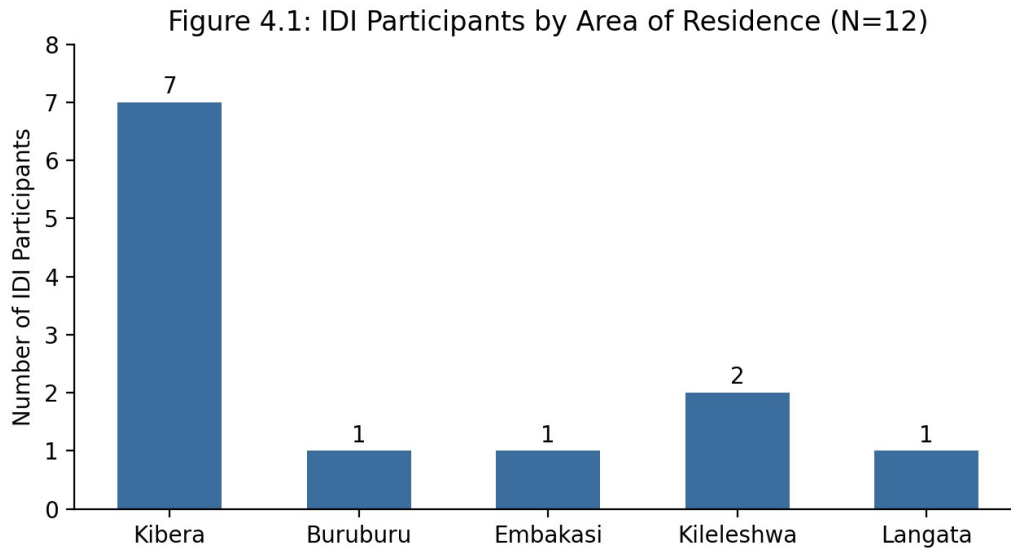
This chapter presents findings from the full data set collected for this study: twelve in-depth interviews (IDIs) and one focus group discussion (FGD) with six additional participants, for a combined total of eighteen adolescent girls and young women (AGYW). As anticipated in Section 3.3, the IDI sample did not remain confined to Buruburu, the estate that had been the study's original point of entry. Because participants were recruited through snowball sampling along their own peer, family, and community networks (Section 3.6), the twelve IDIs ultimately drew on AGYW living in five different parts of Nairobi: Buruburu, Embakasi, Kileleshwa, and Langata, middle-to-upper income residential areas broadly comparable in profile to the estate the study set out to examine, and Kibera, one of Nairobi's largest informal settlements, reached through a separate referral chain. To protect participant identity, all twelve IDI participants are referred to in this chapter using anonymised labels K1 through K12 rather than by name. K1–K7 denote the participants interviewed in Kibera; K8–K12 denote the five participants interviewed in Buruburu, Embakasi, Kileleshwa (two participants), and Langata. FGD participants are not individually labelled, consistent with the aggregated, group-level reporting convention used in the focus group summary; where a single perspective is described, it is attributed to “an FGD participant” rather than to a numbered individual. Sections 4.2 to 4.6 present the IDI findings, organized by geography as an opportunity to examine directly the comparison the study originally set out to make between a middle-income estate and the informal-settlement literature (Section 3.3). Section 4.7 presents the FGD findings in their own right, and Section 4.8 triangulates the two data sources.

4.2 Participant Profiles (In-Depth Interviews)

Table 4.1 summarizes the age, area of residence, and schooling or work status of all twelve IDI participants. Ages ranged from 13 to 21 ($M \approx 18$), and the sample included both in-school and out-of-school AGYW, consistent with the study population defined in Section 3.4.

Table 4.1: IDI Participant Profiles (K1–K7 = Kibera; K8–K12 = Buruburu, Embakasi, Kileleshwa, Langata)

Label	Age	Area of Residence	Schooling / Work Status
K1	19	Kibera	In school; not working; resident 3 months
K2	19	Kibera	University student (1st year); resident 4 years
K3	19	Kibera	University student, first year completed, plus gig work; resident 4 years
K4	19	Kibera	Student, University of Nairobi; resident 4 years
K5	21	Kibera	Out of school (fees); informal trade and salon work; lifelong resident
K6	19	Kibera	Student
K7	19	Kibera	In school; not working; resident 1 year
K8	19	Embakasi	BComm student, Strathmore University
K9	19	Kileleshwa	Not yet in university; describes life as demanding
K10	13	Buruburu	Grade 8 student
K11	18	Langata	Strathmore University student
K12	18	Kileleshwa	High school student



Seven of the twelve IDI participants (58%) were interviewed in Kibera; the remaining five were spread across four middle-to-upper income areas, one or two participants per area.

4.3 Cultural Barriers to SRHR Access (IDI Findings)

Across both the middle-income and informal-settlement participants, the most consistently reported barrier was stigma and the fear of being judged, rather than the physical unavailability of services as such. Participants in Buruburu, Embakasi, Kileleshwa, and Langata described being assumed sexually active or immoral for seeking SRHR information: K8 recounted being told, in effect, “what were you doing, opening your legs instead of opening your books,” and K11 pointed to cultural and religious norms, particularly within Islam, that restrict access based on the gender of the service provider. K2, in Kibera, described the same underlying stereotype in almost identical terms: seeking these services risks being labelled “a prostitute... a slut,” because doing so is read as evidence of low moral standing. Judgmental provider attitudes were raised independently by participants in both groups: K2 described a nurse asking a girl seeking a pregnancy test, “Eeh, ata wewe?” (“Oh, even you?”), and K5 described being lectured with, “You are so young; why are you doing these things? Where is your husband?” K8’s account of clinic staff as “not welcoming, intimidating” echoes this closely.

Financial cost was raised as a barrier in both settings but described differently. K11 and K8 named insufficient funds among several barriers; K5 was more specific, explaining that even when a service is nominally free, the cost of a clinic card or of transport to a clinic “where people don't know you” can be prohibitive, and that this can push girls toward paid chemists instead, where a lack of money for pills or an injection can mean “she ends up doing nothing” and takes on greater risk. This chain of cause and effect, in which one barrier compounds another, is exactly the pattern the interview guide's compounding-difficulty question (Section 2) was designed to surface, and it was articulated most concretely by a Kibera participant rather than by the Buruburu-area group, most of whom (K8, K9, K12) reported not having personally experienced this kind of compounding.

A barrier raised only by the Kibera participants was population density itself. K5 described Kibera as “very crowded,” such that the receptionist at a local clinic “might be my mother's friend or a neighbour,” so that “by evening, the whole neighbourhood will know you went to ask for family planning.” None of the five participants from Buruburu, Embakasi, Kileleshwa, or Langata raised this specific mechanism, suggesting that privacy risk in a dense informal settlement operates differently, and more acutely, than in lower-density middle-income estates. Awareness gaps were also reported on both sides, though with different emphasis: K1 described uneven awareness within Kibera, K3 pointed specifically to a shortage of gynaecologists, and K9 and K12 described limited knowledge of what services exist at all. Peer influence cut both ways: K3 described a friend giving unhelpful advice that discouraged her from seeking medical care, while K4 described peers as the most trusted first stop precisely because “we are the same age... we won't judge each other.”

4.4 Sources of Support and Trust (IDI Findings)

Participants across both groups converged on a common underlying logic for who they trust: sources are trusted less because of their institutional type and more because they are perceived as private and non-judgmental. Family members, especially mothers and aunts, were named as a first port of call by K9, K12, and K11, and K3 described her mother as the person who had actually pointed her toward reliable resources, describing her immediate family as “open-minded” even while extended family held more “backward” views. K6 similarly reported that her family would have no issue with her seeking SRHR information, though judgment would still depend on the specific service sought.

Anonymous digital sources were the other major category of trusted support, raised independently by participants in both areas. K8 described a graded trust in AI tools (“ChatGPT... are liars,” versus higher trust in Gemini and Claude), while K2 described turning first to Google specifically because a search engine “isn't a human; it wouldn't judge me,” and because search history can be deleted. K5 named friends and peers as her first stop, turning to the internet and TikTok only as a fallback, and drew a distinction between trusting online sources for facts and trusting specific friends for real-life experience. K10's reflection on this pattern, that anonymous AI tools solve the judgment problem but introduce an unverified accuracy problem, was echoed structurally by K6, who described cross-checking information found on social media against friends who work as nurses or doctors, since “where there is truth, there is always going to be a bit of false.”

Clinics, hospitals, and chemists featured more prominently, and more ambivalently, as a first stop among the Kibera participants than among the Buruburu-area group. K1 said she trusts a “recognized hospital” specifically; K7 described going to the chemist first, before a hospital, out of fear of cost and of “being told the full reality” there; and K5 named specific providers available locally (AMREF, MSF, public dispensaries, chemists) while noting that awareness of these options is high but access to them, given cost and privacy concerns, remains the harder

part. Taken together, trust was rarely reported as trust in “the clinic” or “the family” as an institution, but in a specific known-safe person or channel within it, for instance K5's account of trusting a friend's referral to “a nice nurse who won't yell at you” over the clinic system in general.

4.5 Social, Familial, and Physical Risks (IDI Findings)

Participants in both groups agreed that a girl found to be seeking SRHR services would generally be assumed to be sexually active, and that this assumption carries social cost: K8 described “very, very big and aggressive” community reactions; K2 described being “shunned, isolated, judged, misunderstood”; and K5 described gossip severe enough to risk being “beaten or chased away” from home. Both groups also converged on a more specific finding: the severity of the reaction depends on the type of service sought. K8, K9, and K11 all reported that contraception draws sharper judgment than general counselling or HIV testing, since contraception is read as direct evidence of sexual activity; none of the Kibera participants were asked this distinction as explicitly, but K7's account, that a family would assume a girl seeking information was “a victim of sexual abuse,” suggests that motive-guessing of this kind is a shared community response across both settings, even where the assumed motive differs.

The clearest divergence between the two groups was in the severity of concrete harm reported. Among the five Buruburu-area participants, no one reported a personal or close secondhand experience of serious harm; K8 and K9 both said they had not personally experienced difficulty, though K8 noted that secrecy is passed down in her family, with her mother and cousin remaining “so secretive” about their own experiences, a pattern K10 characterized as a harm in its own right, since it teaches the next generation that the topic requires hiding by default. Among the Kibera participants, by contrast, two of the more serious harms in the entire dataset were reported. K3 described a classmate who, after being unable to access safe abortion

services, turned to an uncertified provider and suffered severe medical complications, including bleeding and foaming at the mouth. K5 described a friend who was told by a nurse, incorrectly, that an implant would cause future infertility; frightened, she left without protection, became pregnant two months later, and had to drop out of her catering course. These accounts suggest that while the social script around SRHR-seeking, judgment, assumed sexual activity, gossip, is broadly similar across the two settings, the physical stakes of not accessing safe services, and of receiving unsafe or incorrect care when access is restricted, appear substantially higher in Kibera.

4.6 Comparative Reflection: Geography and Socioeconomic Context (IDI Findings)

Because the IDI sample spans both a middle-income cluster of estates and an informal settlement, participants' own comparative reflections (Section 5 of the interview guide) provide a direct, if informal, test of the question the study's methodology was originally designed to ask: whether the cultural barriers documented in the informal-settlement literature extend to a different socioeconomic context, or operate differently there. Notably, participants on both sides of this divide arrived at the same comparison independently. K8, from Embakasi, contrasted an affluent area such as Muthaiga, where a girl would receive “fast,” “efficient” service and “good medicine,” with an informal settlement such as Kibera, where she expected girls to be “dismissed” or told their pain is normal. K5, interviewed in Kibera, made essentially the mirror-image observation without having heard K8's account: girls in areas like Westlands or Kilimani “have money to go to private hospitals where nobody cares about your life... you just pay, get your service, and leave,” whereas in Kibera “poverty makes you exposed” and access to private space is minimal. K2 offered a similar contrast with Kitengela and Nakuru, pointing to lower internet access and awareness within Kibera specifically as a driver of the

gap. At the same time, K5 was careful to note a limit to this divergence: the fear of parents finding out, in her view, “is the same for all Kenyan girls,” regardless of area.

This convergence supports a more precise version of the study's original hypothesis than either group's account would support alone: the core cultural script around SRHR, stigma, assumed sexual activity, family judgment, and harsher reactions to contraception specifically, appears broadly constant across a middle-income estate and an informal settlement, while structural access, provider quality, privacy, and the physical consequences of restricted access vary sharply by socioeconomic context. K10's observation is a useful frame for this finding: what looks like undifferentiated “cultural stigma” at a distance may, on closer inspection, separate into a slower-moving cultural or religious layer and a more tractable structural layer of under-resourced, under-staffed facilities and untrained provider behaviour, and the Kibera data in this chapter, unsafe abortion access, incorrect information from a nurse, crowding that defeats confidentiality, illustrates that second, more fixable layer with particular clarity. Section 4.7 now turns to the focus group discussion, which reached a separate group of six AGYW and offers an independent check on several of these themes.

Table 4.2 collects one representative quotation for each of the main themes identified across the twelve IDIs, to illustrate the findings discussed in Sections 4.3 to 4.6 in participants' own words.

Table 4.2: Representative Quotations by Theme (IDI Participants)

Theme	Participant(s)	Representative Quote
Stigma / assumed sexual activity	K8	<i>“What were you doing, opening your legs instead of opening your books.”</i>
Provider judgment	K2	<i>“Eeh, ata wewe?” (“Oh, even you?”, said by a nurse to a girl requesting a pregnancy test)</i>
Cost as a compounding barrier	K5	<i>“She ends up doing nothing” when she cannot afford pills or an injection.</i>
Density and loss of privacy	K5	<i>“By evening, the whole neighbourhood will know you went to ask for family</i>

		<i>planning.”</i>
Trust in anonymous digital sources	K2	<i>A search engine “isn't a human; it wouldn't judge me,” and search history can be deleted.</i>
Geography and service quality	K8	<i>Girls in an affluent area get “fast,” “efficient” service and “good medicine.”</i>

4.7 Focus Group Discussion Findings

One focus group discussion (FGD) was conducted with six AGYW, following the FGD guide included as Appendix B. Unlike the IDIs, FGD contributions are reported here as they were recorded in the discussion summary: at the level of the group rather than as individually numbered participants. The discussion was organized around the same five thematic areas as the IDI guide, plus a cross-cutting set of observations flagged by the participants themselves.

4.7.1 General Perceptions of SRHR Services

FGD participants largely endorsed SRHR services in principle, describing them as necessary, confidential, and protective, while several admitted that in practice they only seek care when symptomatic rather than proactively, even while agreeing that preventive visits are likely more beneficial given that some conditions present no early symptoms. A recurring point was that SRHR is widely misread as synonymous with sexual activity: one participant described having to explain to a peer that seeking SRHR services does not imply sexual activity, since the field also covers periods, contraception, and general reproductive health regardless of sexual history. Negative provider interactions surfaced early in the discussion, including one account of a urinary tract infection being misdiagnosed as a sexually transmitted infection, which participants linked to a broader tendency among providers to assume that anything reproductive-health-related is sexual in origin.

4.7.2 Cultural Barriers and Community Attitudes

The group identified a wider spread of barriers than any single IDI participant raised alone. Participants from FGM-practicing communities described complications from FGM, pain, bleeding, and difficulty in childbirth, as generating a shame that in turn discourages related care-seeking, so that the practice compounds rather than only causes physical harm. Family communication barriers were widely shared: participants described being unable to discuss sexual or reproductive health with parents for fear of judgment, distrust of aunts specifically, and discomfort discussing periods or sex with fathers. Provider dismissiveness recurred as a theme, with several participants describing gynaecologists who characterized severe pain as “normal period pain,” in at least one case delaying diagnosis of a more serious condition for years. Religious and cultural teachings discouraging contraception specifically were also raised, as was one secondhand account, relayed from a medical professional, of severe childbirth complications in a patient from a community practicing genital sewing as a form of FGM. One participant observed that the general absence of health professionals from community spaces signals to boys and young men that these topics are not taken seriously, and that gender-segregated health education leaves boys without an equivalent peer support structure.

4.7.3 Sources of Support and Trust

Formal healthcare was not the dominant information source for most FGD participants. In descending order of how frequently they were raised, participants named AI tools such as ChatGPT (one participant said she relies on AI more than she would confide in a sibling, while explicitly noting that AI “can be manipulated” and is not fully reliable), online communities and social media (valued for lowering the barrier to learning relative to an in-person conversation, and credited with shifting norms around generational trauma and reproductive health), peer networks including friends studying medicine, and gynaecologists, whose quality

participants described as inconsistent, with some dismissive of concerns and others attentive. Several participants also described routinely self-diagnosing based on online research or past experience, with explicit awareness of misdiagnosis risk, for instance confusing a yeast infection with a urinary tract infection. One participant disclosed that she is personally developing an AI-based menstrual symptom triage tool, motivated by her own experience of having her symptoms dismissed by providers.

4.7.4 Social, Familial, and Physical Risks

The FGD produced a more granular account of how stigma varies by service type than any single IDI did. Contraception, especially visible methods such as implants, was linked to being labelled “immoral” and to gossip. HIV testing was described as carrying the highest stigma of all, with an automatic assumption of promiscuity. Counselling was discussed as comparatively less stigmatized, though still not neutral. A gendered double standard also emerged: participants noted that women can obtain monthly contraceptives from a chemist under a vague pretext, such as citing heavy periods, without much suspicion, while men purchasing condoms are also judged despite condoms serving a protective function the group otherwise endorsed; one participant noted this contradicted her initial assumption that only women face judgment around SRH purchases. The image of pads wrapped in newspaper recurred multiple times in the discussion as shorthand for how menstruation-related purchases are treated as something to conceal, even though menstruation is universal among women.

4.7.5 Comparative Reflection Across Communities

FGD participants agreed that the overall pattern, stigma, family silence, and provider dismissiveness, is broadly shared across communities but is worse in specific contexts; pastoralist and some Muslim communities were named as facing compounded barriers, including FGM and early marriage layered on top of the general stigma described elsewhere in

the discussion. Participants also expressed cautious optimism, pointing to social media as a space where mothers and younger women are increasingly discussing generational trauma and reproductive health openly, which several felt was slowly shifting norms, albeit unevenly across communities.

4.7.6 Cross-Cutting Observations from the FGD

Beyond the five thematic sections, the FGD summary flags several observations that cut across them. Participants consistently endorsed proactive, preventive SRHR use in the abstract while describing their own behaviour as reactive and symptom-triggered, a behaviour-belief gap treated in the discussion as distinct from stigma-driven avoidance. One participant reported feeling less safe in a one-on-one session with an unfamiliar provider than in a group setting, cutting against the common assumption that privacy is the primary condition for disclosure comfort. Participants also converged on a service-specific stigma gradient, HIV testing carrying the most stigma, followed by contraception, with counselling the least stigmatized, and on the observation that informal and digital channels function as the primary source of SRHR information for this group, with formal healthcare providers treated as secondary or a last resort. Finally, the group itself proposed that future sessions move from experience-sharing toward solution-building, and specifically recommended including younger girls and male participants in future rounds of data collection.

4.8 Triangulating the IDI and FGD Findings

The two data sources converge on several core findings, which strengthens confidence in them. Both the IDIs and the FGD identify stigma and fear of judgment, rather than the physical unavailability of services, as the central barrier to SRHR access, and both identify family communication, particularly discomfort discussing SRHR with parents or male relatives, as a specific mechanism through which that stigma operates. Both sources also independently

identify informal and digital channels, AI tools, social media, and trusted peers, as functioning ahead of formal healthcare providers as a first-line source of information, and both flag the accuracy risk this carries: K10 and K6 raised this concern from the IDI side (Section 4.4), and the FGD participant developing an AI triage tool raised it independently from the group side (Section 4.7.3). Both sources further agree that the severity of social reaction is service-specific rather than uniform: the IDI participants (Section 4.5) reported contraception drawing harsher judgment than counselling, and the FGD refines this into a more precise three-tier gradient, HIV testing above contraception above counselling, which is a more actionable finding for intervention design than either data source offers alone.

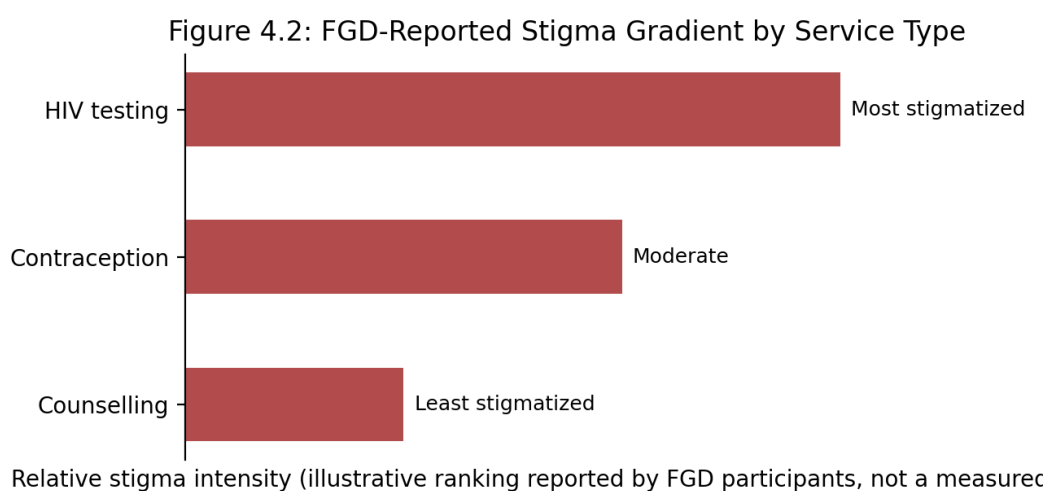


Figure 4.2 illustrates the ordinal stigma gradient the FGD converged on (Section 4.7.4); it is a qualitative ranking reported by participants, not a measured quantity.

The FGD also surfaced two themes that did not emerge clearly from the twelve IDIs. First, FGM and related practices, including one account of severe childbirth complications following genital sewing, were raised only in the FGD; no IDI participant, including those interviewed in Kibera, raised FGM directly, which may reflect the specific communities represented in the FGD rather than a genuine absence of the issue in the wider study population. Second, the FGD's behaviour-belief gap, endorsing preventive care in principle while practising reactive,

symptomatic care in fact, is a more precise articulation of a pattern only hinted at in the IDIs (for instance K1's and K3's comments on uneven awareness in Section 4.3), and is worth treating as a distinct barrier in its own right rather than folding it into stigma, consistent with the FGD summary's own analysis. Conversely, the concrete physical harms documented in the Kibera IDIs, unsafe abortion complications and a pregnancy following incorrect provider advice (Section 4.5), were not mirrored by an equivalent account in the FGD, though the FGD's description of gynaecologists delaying diagnosis by years is a comparable instance of serious harm arising from provider dismissiveness. Read together, the two data sources support a consistent overall picture: a broadly shared cultural script of stigma and family silence around SRHR, operating with varying severity depending on socioeconomic context, service type, and specific community practice, and channelling AGYW toward informal, unverified sources of information and support rather than toward formal healthcare.

4.9 Summary of Key Findings

Six findings stand out from this chapter. First, the realized IDI sample was geographically broader than planned, spanning Buruburu, Embakasi, Kileleshwa, Langata, and Kibera, which, rather than undermining the study, allowed the middle-income-versus-informal-settlement comparison motivating the original site selection (Section 3.3) to be examined directly. Second, stigma and fear of judgment, not the physical unavailability of services, was the barrier raised most consistently across both the IDIs and the FGD. Third, the FGD refines the IDI finding on service-specific stigma into a three-tier gradient, HIV testing carrying the most stigma, followed by contraception, then counselling, which offers a more actionable basis for targeted destigmatization messaging than treating SRHR-seeking as uniformly stigmatized. Fourth, trust in an information source depended more on perceived privacy and non-judgment than on institution type, and both data sources independently identify informal and digital channels, AI

tools, social media, and trusted peers, as more central to AGYW information-seeking than formal healthcare providers, with both sources also flagging the accuracy risk this carries. Fifth, while the cultural script around SRHR-seeking was broadly similar across areas and data sources, the concrete physical and financial stakes of restricted access were markedly higher among the Kibera IDI participants and among the FGD accounts of FGM-related complications and delayed diagnosis, than among the middle-income IDI participants. Sixth, the FGD identified a behaviour-belief gap, participants endorsing preventive SRHR use in principle while describing their own care-seeking as reactive, that is analytically distinct from stigma-driven avoidance and was not clearly visible in the IDI data alone, underscoring the value of combining individual and group-level methods in this study.

CHAPTER FIVE

DISCUSSION AND ANALYSIS

5.1 Introduction

This chapter interprets the findings presented in Chapter Four in relation to the literature reviewed in Chapter Two, and considers what the combined IDI and FGD dataset adds to, confirms, or complicates in that literature. It closes with a discussion of the limitations of the study.

5.2 Stigma as the Central, Cross-Cutting Barrier

The single most consistent finding across all eighteen participants, IDI and FGD alike, is that stigma and fear of judgment, rather than the physical unavailability of services, is the primary barrier to SRHR access. This is broadly consistent with the wider literature on AGYW and SRHR in Sub-Saharan Africa reviewed in Chapter Two, which similarly identifies moral policing and provider bias as central obstacles rather than infrastructure alone (Solo & Festin, 2019; Chakupewa & Mmari, 2021). What the present study adds to this picture is a more specific account of how stigma is enacted day to day: through a nurse's remark, through the assumption that a girl buying contraception intends to have sex, through gossip that a dense, low-privacy neighbourhood makes almost impossible to contain. Where the literature has tended to describe stigma as a general climate, the participants in this study described it as a set of specific, repeatable interactions, a finding that has direct implications for intervention design, since specific interactions are more amenable to provider training than a diffuse cultural climate is.

5.3 A More Precise Account of Service-Specific Stigma

The FGD's three-tier stigma gradient, HIV testing carrying the most stigma, followed by contraception, then counselling, refines a pattern that individual IDI participants (K8, K9, and K11 in particular) described only in binary terms, contraception versus everything else. This is a more precise finding than the existing literature typically offers, since much of the SRHR literature treats stigma as attaching to the field as a whole rather than differentiating by service type. Read alongside Solo and Festin's (2019) review of provider bias in family planning, which documents that providers themselves apply different levels of scrutiny depending on the method requested, the present findings suggest that this service-specific gradient operates on both sides of the provider-client relationship: providers appear to scrutinize certain services more than others, and communities appear to judge AGYW who seek them more harshly in direct proportion. This convergence between the provider-side literature and the community-side accounts gathered here strengthens the case that destigmatization messaging should be targeted by service type rather than applied uniformly across SRHR.

5.4 Structural Access Varies More Than Cultural Script Does

Chapter Four's geographic comparison, drawn from the unplanned but analytically useful spread of the IDI sample across Buruburu, Embakasi, Kileleshwa, Langata, and Kibera, supports a finding that is easy to state but not always made explicit in the literature: the underlying cultural script of judgment and family silence around SRHR appears broadly constant across a middle-income estate and an informal settlement, while structural access, provider quality, privacy, and the physical consequences of restricted access vary sharply by socioeconomic context. This is consistent with, and gives grounded texture to, Beguy et al.'s (2009) and Ziraba et al.'s (2018) observation that adolescent SRH research in Nairobi has concentrated disproportionately on informal settlements, since the present findings suggest that doing so risks understating how similar the underlying stigma dynamics are across settings,

even as it correctly captures that the stakes of restricted access are higher in under-resourced areas. The two most serious harms recorded in the entire dataset, an unsafe abortion and a pregnancy following incorrect provider advice, both came from Kibera participants, while the FGD's account of a diagnosis delayed for years by a dismissive gynaecologist shows that serious harm from provider dismissiveness is not confined to informal settlements either. Taken together, this suggests socioeconomic context shapes the severity and physical stakes of restricted access more than it shapes whether stigma exists in the first place.

5.5 Informal and Digital Channels Are Filling a Trust Gap

Both data sources independently show formal healthcare providers functioning as a secondary or last-resort source of SRHR information, with AI tools, social media, and trusted peers doing much of the work instead. This is a notable departure from the assumption, common in SRHR programming, that improving clinic-based information provision is sufficient to close information gaps. If AGYW are not going to clinics first, clinic-based education campaigns will reach them only after they have already formed views, sometimes inaccurate ones, elsewhere. At the same time, participants across both the IDIs and the FGD showed real awareness of the accuracy risk this carries, several explicitly distrusted AI tools even while using them, and several described cross-checking information against a trusted friend in the medical field. This suggests AGYW are not passive consumers of unreliable information so much as they are making a reasoned trade-off between accuracy and judgment-free access, a trade-off that clinic-based services could shift by becoming demonstrably less judgmental, rather than simply more available.

5.6 Limitations

Several limitations qualify these findings. First, the sample was recruited through snowball sampling along pre-existing peer and family networks rather than through random or stratified

selection, so the eighteen participants cannot be assumed to represent AGYW in Nairobi more broadly, and the geographic spread that this chapter and Chapter Four treat as analytically useful was a byproduct of how referrals happened to travel, not a designed comparison. Second, the study is based on a single FGD of six participants and twelve IDIs; both are small numbers for a topic as internally varied as SRHR access, and a larger sample would likely surface barriers not captured here, such as those specific to communities practicing FGM, which appeared only in the FGD and in none of the twelve IDIs. Third, the study did not include key informant interviews with healthcare providers, parents, or community gatekeepers; Objective 2, concerning provider bias, is therefore addressed only through AGYW's own accounts of provider behaviour, not through providers' own perspectives, which may differ. Fourth, one IDI transcript (K10) departed noticeably in style from the other eleven, reading more like third-person analytical commentary than a first-person response; this transcript was retained and reported as given, but its answers should be treated with more caution than the others. Fifth, the study proceeded without formal institutional ethics clearance, as noted in Section 3.8, relying instead on community gatekeeper permission and the consent and protection procedures described there; this is appropriate for a bootcamp training exercise but would need to be addressed before any future or published iteration of this work. Finally, all interviews and the focus group were conducted and analysed within a five-day data collection window, which constrained both the sample size and the depth of follow-up possible with any individual participant.

CHAPTER SIX

CONCLUSION AND RECOMMENDATIONS

6.1 Conclusion

This study set out to investigate the cultural factors that influence SRHR access among AGYW aged 10 to 24 in Nairobi. Across twelve in-depth interviews and one focus group discussion, spanning both a cluster of middle-income estates and an informal settlement, stigma and fear of judgment emerged as the central barrier to access, operating through specific, repeatable interactions with family members, peers, and healthcare providers rather than through a vague cultural climate alone. The severity of that stigma varies by service type, is highest for HIV testing and lowest for counselling, and its physical and financial consequences vary sharply by socioeconomic context, even though the underlying cultural script appears broadly constant across the communities studied. In the absence of trusted, non-judgmental formal services, AGYW are turning to AI tools, social media, and peer networks for SRHR information, a pragmatic response to a real trust gap, but one that carries its own accuracy risks. These findings suggest that closing this gap will require interventions that address provider behaviour and community norms specifically, rather than treating service availability alone as the constraint.

6.2 Recommendations

Based on these findings, five recommendations follow. First, healthcare providers serving AGYW should receive targeted training on non-judgmental communication, with particular attention to HIV testing and contraception counselling, the two services this study found to carry the most stigma; the goal should be to make clinics demonstrably less judgmental, not merely more available. Second, community and family-facing education campaigns should explicitly address the common misconception that SRHR-seeking implies sexual activity, since

this misconception, raised independently by both IDI and FGD participants, appears to drive a substantial share of the judgment AGYW anticipate and experience. Third, given that AGYW already rely heavily on AI tools and social media for SRHR information, organizations working in this space should consider partnering with, or building, digital tools with embedded accuracy safeguards, meeting AGYW in the channels they already trust rather than competing with them. Fourth, confidentiality-focused service design, private entrances, discreet packaging, staff continuity, matters more in dense, low-privacy settings such as Kibera than in lower-density estates, and providers operating in such settings should treat privacy protection as a distinct design problem rather than an extension of general service quality. Fifth, future research on this topic should expand the sample beyond what a five-day bootcamp timeline allows, include key informant interviews with providers and gatekeepers, and, as the FGD participants themselves recommended, extend data collection to younger adolescents and to male peers, whose absence from formal SRHR education was raised as a concern in its own right during the focus group discussion.

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Sample Data Collection Tools

A. Semi-Structured In-Depth Interview (IDI) Guide

Introduction (read aloud to the participant before starting): “Thank you for agreeing to speak with me today. My name is [Interviewer Name], and I am part of a research team looking into the experiences of young women like yourself in accessing sexual and reproductive health information and services here in [name of participant's community]. There are no right or wrong answers; I am simply interested in your own experience and views. You do not have to answer any question you do not want to, and you can stop at any time. Everything you share will be kept confidential and will not be linked back to your name. Do you have any questions before we begin?” [Obtain verbal consent or assent, using the appropriate consent or assent form, before proceeding.]

Section 1: Biography

- Can you tell me a little about yourself, for example your age, whether you are currently in school, and what life is generally like for you here in [name of participant's community]?

Section 2: Cultural Barriers to SRHR Access

- When you think about sexual and reproductive health services in this community, what comes to mind first? (Probe, only if needed after the participant has answered in their own words: clinics, counselling, information sessions.)
- What, if anything, makes it difficult for girls and young women your age to access these services? (Probe: Are there specific beliefs, traditions, or expectations in the community that make this harder?)

- Sometimes one difficulty can make another one worse, for example, if not being able to afford a service also makes it harder to keep it private from family. Can you think of a time this happened, either to you or to someone you know?

Section 3: Sources of Support and Trust

- When you or girls you know need information or help related to SRHR, where do you usually go first? (Probe: friends, family members, religious leaders, clinics, community health workers, social media, teachers.)
- How much do you trust the source(s) you just mentioned, and why?

Section 4: Social, Familial and Physical Risks

- What do you think would happen, within your family or community, if people found out a girl was seeking SRHR services or information? (Probe: social risks, family reaction, concerns about physical safety; and whether the reaction might differ depending on the type of service being sought, for example contraception versus HIV testing versus general counselling.)
- Have you, or someone you know, ever faced a difficult situation after seeking this kind of help? Could you tell me about it, only if you feel comfortable doing so?

Section 5: Comparative Reflection and Closing

- Have you heard whether things are similar or different for girls in other parts of Nairobi or other communities?
- Is there anything else about this topic that you think is important for us to know, that we have not already discussed?

Closing script: “Thank you so much for sharing your experience with me today. If anything we discussed has brought up difficult feelings, here is the contact information for [name of local

support service], who you can reach for support. Do you have any questions for me before we finish?”

B. Focus Group Discussion (FGD) Guide (6 participants)

Ground rules (read aloud): “Thank you all for joining today. This discussion is about the experiences girls and young women in this community have around sexual and reproductive health services. There are no right or wrong answers. Please respect each other's privacy; what is shared here should stay here. You do not have to share anything you are not comfortable with.”

Section 1: General Perceptions

- In general, what do girls and young women in this community think about seeking SRHR services?

Section 2: Cultural Barriers to SRHR Access

- What cultural barriers or community attitudes make it harder for girls to access these services?

Section 3: Sources of Support and Trust

- Where do girls in this community usually go for SRHR information or help?

Section 4: Social, Familial and Physical Risks

- What do people in this community generally think happens to a girl if others find out she sought SRHR services? (Probe: does this differ depending on the type of service being sought, for example contraception versus HIV testing versus general counselling?)

Section 5: Comparative Reflection and Closing

- Have you heard whether the situation is similar or different in other communities?

- Is there anything else the group feels is important to add?

C. Adult Consent Form

For participants aged 18 to 24.

Study title: Investigating Cultural Factors Influencing Sexual and Reproductive Health Rights (SRHR) Service Access Among Adolescent Girls and Young Women (AGYW), Aged 10-24, in Buruburu, Nairobi, Kenya

Researchers: Joy Chepkorir Bett, Lvy Oguye, and Denis Njoroge (RCA Bootcamp research team)

What this study is about

We are a small student research team looking into the cultural factors that make it harder, or easier, for young women in Buruburu to access sexual and reproductive health services and information. We want to understand your own experiences and views on this topic.

What participation involves

If you agree to take part, you will be asked to join either a one-on-one interview (about 45 to 60 minutes) or a small group discussion with other participants (about 60 to 90 minutes), not both. You will be asked about your experiences and opinions regarding SRHR services, cultural barriers, sources of support, and any risks you associate with seeking these services. With your permission, the session will be audio recorded so we can accurately capture what you share.

Voluntary participation and right to withdraw

Taking part is entirely voluntary. You do not have to answer any question you do not want to, and you may stop or leave at any time, including after the session has taken place, without giving a reason and without any negative consequence to you.

Confidentiality

You will not be identified by name in any transcript, recording label, or written report. You will be assigned a code instead. Recordings and transcripts will be stored securely on password-protected devices accessible only to the research team, and any quotations used in the final report will be anonymized so that they cannot identify you.

Possible discomfort and support

Some questions in this study are personal and may bring up difficult feelings. You are free to skip any question or stop the session at any time. If you would like to talk to someone afterward, free counselling support is available at: CITAM Buruburu Counseling Center: 0758009009, CITAM Buruburu, Buruburu Phase 5.

Questions

If you have any questions about this study, now or later, you can contact any member of the research team: Joy Chepkorir Bett, +254 700 024985; Lvy Oguye, +254 731 696480; Denis Njoroge, +254 742 445642.

Statement of Consent

I have read, or had read to me, the information above. I understand what my participation involves, and I understand that I may withdraw at any time without penalty. I voluntarily agree to take part in this study.

Participant's printed name: _____

Participant's signature: _____ Date: _____

I agree to this session being audio recorded: Yes ☐ No ☐

Researcher's name: _____

Researcher's signature: _____ Date: _____

D. Minor Assent Form

For participants under 18 years of age.

We are a small student research team, and we want to learn about the experiences of young women in Buruburu when it comes to sexual and reproductive health, things like clinics, information, and support. We would like to ask you some questions about this.

What would happen

If you say yes, you will either talk with one of us alone for about 45 to 60 minutes, or join a small group discussion with other girls for about 60 to 90 minutes, not both. We will ask about your experiences and thoughts on this topic. There are no right or wrong answers. If it is okay with you, we will record the conversation using an audio recorder so we remember what you said correctly.

It's your choice

You do not have to say yes. Even if your parent or guardian has already said it is okay, it is still your choice too. You can skip any question you do not want to answer, and you can stop at any time, even after we've started, without getting into any trouble.

Keeping things private

We will not use your name anywhere. We will give you a code instead, so no one will know it was you who said what. What you tell us will be kept safe and only our small research team will see it.

If something feels hard

Some questions might make you think about things that feel difficult. That's okay, you can skip them or stop any time. If you want to talk to someone afterward, there is a free place you can go: CITAM Buruburu Counseling Center: 0758009009, CITAM Buruburu, Buruburu Phase 5.

Questions

If you have any questions, you can ask us anytime, before, during, or after. You can also ask your parent or guardian to contact us: Joy, +254 700 024985; Lvy, +254 731 696480; Denis, +254 742 445642.

Do you agree to take part?

Please tell us, or mark below:

Yes, I want to take part ☐ No, I do not want to take part ☐

Your name: _____ Your age: _____

Your signature (or mark, if you prefer): _____ Date: _____

I agree to this conversation being recorded: Yes ☐ No ☐

Researcher's name: _____

Researcher's signature: _____ Date: _____

E. Parent / Guardian Consent Form

For participants under 18 years of age.

Study title: Investigating Cultural Factors Influencing Sexual and Reproductive Health Rights (SRHR) Service Access Among Adolescent Girls and Young Women (AGYW), Aged 10-24, in Buruburu, Nairobi, Kenya

Researchers: Joy Chepkorir Bett, Lvy Oguye, and Denis Njoroge (RCA Bootcamp research team)

What this study is about

We are a small student research team looking into the cultural factors that make it harder, or easier, for young women in Buruburu to access sexual and reproductive health services and information. This letter is to ask your permission for your child to take part.

What participation involves

If you give permission, and your child also agrees, she will be asked to join either a one-on-one interview (about 45 to 60 minutes) or a small group discussion with other participants (about 60 to 90 minutes), not both. She will be asked about her experiences and opinions regarding SRHR services, cultural barriers, sources of support, and any risks she associates with seeking these services. Questions are age-appropriate and she will not be asked to disclose specific personal experiences of abuse or risk. With permission, the session will be audio recorded so we can accurately capture what is shared.

Voluntary participation and right to withdraw

Participation is entirely voluntary, for both you and your child. Your child does not have to answer any question she does not want to, and either of you may withdraw her from the study at any time, including after the session has taken place, without giving a reason and without any negative consequence.

Confidentiality

Your child will not be identified by name in any transcript, recording label, or written report. She will be assigned a code instead. Recordings and transcripts will be stored securely on password-protected devices accessible only to the research team, and any quotations used in the final report will be anonymized so that they cannot identify her.

Possible discomfort and support

Some questions in this study are personal and may bring up difficult feelings. Your child is free to skip any question or stop the session at any time. Sessions with participants under 16 will take place with a trusted community adult present. If your child would like to talk to someone afterward, free counselling support is available at: CITAM Buruburu Counseling Center: 0758009009, CITAM Buruburu, Buruburu Phase 5.

Questions

If you have any questions about this study, now or later, you can contact any member of the research team: Joy Chepkorir Bett, +254 700 024985; Lvy Oguye, +254 731 696480; Denis Njoroge, +254 742 445642.

Statement of Consent

I have read, or had read to me, the information above. I understand what my child's participation involves, and I understand that either of us may withdraw her at any time without penalty. I voluntarily give permission for my child to take part in this study.

Child's name: _____ Child's age: _____

Parent/Guardian's printed name: _____

Relationship to child: _____

Parent/Guardian's signature: _____ Date: _____

I agree to this session being audio recorded: Yes ☐ No ☐

Researcher's name: _____

Researcher's signature: _____ Date: _____

F. IDI Participant Summaries – K8–K12 (Raw Data)

This appendix reproduces the interview summaries for participants K8 through K12 as recorded by the research team, organized by interview guide section and question, with participant labels anonymised to match the labelling used throughout this report.

Section 1: Biography

Can you tell me a little about yourself, for example your age, whether you are currently in school, and what life is generally like for you here in Kenya?

K8: K8 is 19 years old and lives in Embakasi, Nairobi. She studies BComm at Strathmore University. Regarding life in Kenya, K8 describes it as "quite tiring and stimulating... overstimulating and demanding".

K9: K9 is 19 years old and lives in Kileleshwa, Nairobi. She is yet to join university. Life in Kenya as described by K9 is quite demanding and stressful in terms of the economy and general wellbeing of the nation.

K10: K10 is a 13 year old living in Buruburu, Nairobi. She is a grade 8 student. Regarding life in Kenya, K10 says it has ups and downs especially for older people.

K11: Crystal K11 is 18, living in Langata, Nairobi and a Strathmore University student; described life in Nairobi as generally hard.

K12: K12 is an 18 year old girl, living in Kileleshwa, Nairobi and currently still in high school. Life is generally okay for her. However, she says it's financially difficult for many people at the moment. She has nothing to complain about.

Section 2: Cultural Barriers to SRHR Access

When you think about sexual and reproductive health services in this community, what comes to mind first? (Probe, only if needed after the participant has answered in their own words: clinics, counselling, information sessions.)

K8: On SRHR services: K8 describes SRHR services as involving "creating self awareness among people about the importance of sexual health" When asked where people access these services, she identifies clinics as the main source

K9: She describes the SRHR services as very involved towards the community and help people understand their sexual and reproductive health. K9 identifies local clinics as the main source for such sessions where most people seek care and guidance.

K10: She describes the SRHR services "clinic" being the reflexive answer, while true structurally, is a bit of a mismatch with reality, clinics are the intended access point but not

necessarily where people feel safe going, which is exactly the tension your later questions are designed to surface.

K11: She said hospitals come to mind first, noting that she is aware of few other SRHR services beyond that.

K12: The stigmatization that comes with it. Especially because there is not much awareness and people often think such services should only be rendered when one is unwell.

What, if anything, makes it difficult for girls and young women your age to access these services? (Probe: Are there specific beliefs, traditions, or expectations in the community that make this harder?)

K8: Barriers to access - K8 identifies several barriers: High risk of HIV or pregnancy, Lack of education about the importance of sexual health, Insufficient funds and Religious beliefs (Christianity and Islam) impact access.

K9: From her perspective she believes that sometimes not everyone feels safe accessing them because of stigma/fear of being judged or rather being seen as sexually active (for the unmarried and young women) when in reality for general checkup and the betterment of one's health. Moreover the limited knowledge about the services available.

K10: Her honest view: stigma/judgment is the more tractable barrier of the two categories (structural vs. affective). Cost and religious belief move slowly. Provider behavior and confidentiality norms can be changed faster with training and policy so if she had to prioritize an intervention, she'd start there.

K11: She identified financial constraints as a major barrier. She also pointed to a lack of parental support and awareness, as well as cultural beliefs, particularly within Islam, that restrict access due to the gender of service providers.

K12: She says the massive stigma in the community towards SRHR brings out fear in girls making them hesitant to look for these services. She also says that in rural areas, such services are inaccessible.

Sometimes one difficulty can make another one worse, for example, if not being able to afford a service also makes it harder to keep it private from family. Can you think of a time this happened, either to you or to someone you know?

K8: Compounding difficulties: When asked about situations where one difficulty makes another worse (e.g., lack of funds forcing family involvement), K8 states "Not really. Our household will never...", indicating she hasn't personally experienced this, though the response is incomplete.

K9: When asked about these situations such as lack of funds, K9 states that she has not quite personally experienced this as well.

K10: She'd expect this question to consistently under-elicite real answers in a first-meeting interview format, regardless of who's answering. It's asking someone to admit a personal vulnerability. However, she said lack of funds is normally the issue in Kenya or rather Nairobi.

K11: This was not directly addressed in her response.

K12: She doesn't have any experience of such or had a close friend with this encounter, so no. However, she says that financial issues can cause this since you are forced to beg other family members or neighbours.

Section 3: Sources of Support and Trust

When you or girls you know need information or help related to SRHR, where do you usually go first? (Probe: friends, family members, religious leaders, clinics, community health workers, social media, teachers.)

K8: Information sources: K8 indicates that girls prefer anonymous sources like AI tools rather than approaching family or healthcare workers, citing "high risk of judgment, of lectures, of long process of words".

K9: Information sources: K9 indicates she and the girls she knows often seek guidance from their family members such as their mothers, aunts or any female guardian who they are close to. This is because they offer some sense of comfort to them and the risk of judgement is lower than from healthcare workers.

K10: Her honest take is that turning to family is probably the healthier long-term pattern if the family member is actually knowledgeable, because it keeps the person embedded in a real support relationship. Turning to anonymous AI tools solves the judgment problem but introduces an accuracy problem - nobody's checking whether the information given is medically sound. She flags that as a genuine risk worth taking seriously, not just an interesting data point.

K11: She said she usually turns to her mother or social media first.

K12: She says she trust her trusted family most, that is, her close cousins and a few aunties. Also, she trusts social media as a source of information on SRHR services.

How much do you trust the source(s) you just mentioned, and why?

K8: Trust levels: ChatGPT: Low trust - "are liars. High, high chances". Gemini and Claude AI: Higher trust - "have proved their point" Overall AI trust: "50/50% Of AI is doing their work". Why not clinics: K8 explains clinics are intimidating because healthcare workers are

judgmental. She gives an example: a nurse might say "What were you doing? Opening your legs instead of opening your books". She describes clinic services as "not welcoming, intimidating".

K9: She states "I trust them very much as I feel that since it's coming from a person you trust it's not really embarrassing and you will still get the guidance you need without judgement."

K10: She thinks discernment between different AI tools or distrust is a reasonable instinct. But she also says, for anything with real clinical stakes (dosing, drug interactions, symptoms that need triage). She doesn't think any chatbot should be someone's only source, regardless of which one.

K11: She trusts her mother because of her experience and judgment. She trusts social media more cautiously and tends to verify what she finds there independently.

K12: She trusts social media because the different experiences of girls on platforms like Tiktok and Instagram help in decision making or learning. She also says that with family there is just an obvious trust put to them especially with close ones such as mothers and sisters.

Section 4: Social, Familial and Physical Risks

What do you think would happen, within your family or community, if people found out a girl was seeking SRHR services or information? (Probe: social risks, family reaction, concerns about physical safety; and whether the reaction might differ depending on the type of service being sought, for example contraception versus HIV testing versus general counselling.)

K8: Community reactions: When a girl seeks SRHR services, the community assumes "they want sex only". K8 notes that "if they see you buying those things. Just know you're going to do the thing". Type of service matters: K8 confirms reactions differ by service type: "Of course there'll be different reactions" depending on whether it's contraception versus HIV testing

versus counseling. Severity of reactions: K8 states there will be "Very, very big and aggressive" reactions.

K9: She states that many people would assume she's sexually active, even if she was just looking for information. There would probably be gossip, and some people might judge her or call her disrespectful ; this often leads to a lot of stigmatization. Many girls may avoid seeking these services because they don't want people talking about them. If she was just going for counselling or to ask questions, some people might not think it's a big issue. But if they knew she was getting contraception, many would immediately assume she's planning to have sex, and that would attract a lot of criticism.

K10: Her view is that the fact reactions can vary by type of service (contraception vs. HIV test vs. general counseling) is a really important nuance that a lot of public-facing SRHR messaging misses - treating "seeking any SRHR service" as one uniform risk probably discourages some help-seeking that wouldn't actually provoke the worst reactions.

K11: She said it would often be assumed she was sexually active, which could bring judgment and social stigma. She noted that reactions differ depending on the type of service sought; seeking counseling is generally more accepted, while seeking contraception carries more stigma.

K12: She thinks they'd be accommodating and willing to help. However in the community at large eg neighbours, there may be some judgement dependent on the situation.

Have you, or someone you know, ever faced a difficult situation after seeking this kind of help?
Could you tell me about it, only if you feel comfortable doing so?

K8: Personal experience: K8 has not personally experienced difficult situations: "Not really. I think I'm a homebody, so I've not really experienced such". However, she notes her mother and

cousin are "so secretive" and "keep their stories to themselves," indicating secrecy affects others in her community

K9: Personal experience: K9 has not also experienced such difficult situations.

K10: She thinks secrecy passed down through a family is worth taking seriously as its own kind of harm. It teaches the next generation that this topic requires hiding by default, independent of whether anything went wrong.

K11: She said she has not personally faced a difficult situation after seeking this kind of help.

K12: She says she doesn't think she has heard of any.

Section 5: Comparative Reflection and Closing

Have you heard whether things are similar or different for girls in different parts of Nairobi or different communities?

K8: Disparities by location and class: K8 identifies significant differences based on social class and location. She explains that a girl in Kibera would receive different services than a girl in Muthaiga (an affluent area). Specifically: Girls in affluent areas get "fast service," "efficient service," and "good medicine". However, Girls in Kibera might be "dismissed" or told their pain is normal

K9: Her view is that it varies depending on the community. In some parts of Nairobi, especially where people are more educated or where there are youth-friendly health services, girls may feel a bit more comfortable seeking SRHR information and services. In other communities, especially where cultural or religious beliefs are very strong, girls may face more judgment and stigma.

K10: "Cultural stigma" is sometimes actually an "under-resourced facility with burned-out staff - parents and grandparents," which is a different and more fixable problem than shifting deep-seated cultural attitudes. It affects different communities in different ways.

K11: She believes there is greater stigma and embarrassment around this topic in other parts of Nairobi that she views as less modernized. She also noted that girls raised without their mothers tend to face more awkward situations.

K12: She says that most experiences are different across different parts of the country since we live as different communities in Nairobi and most older people are still deep in their cultural views of secrecy. She says also having people of different religious backgrounds plays part.

Is there anything else about this topic that you think is important for us to know, that we have not already discussed?

K8: Additional important points: K8 emphasizes the critical need for comprehensive sex education. She notes: Parents rarely discuss sex and the importance of protection, HIV cases in Kenya are "too many". Sex education should focus on "how to manage it" (safe sexual activity) rather than abstinence-only approaches. Even though many young people don't follow the guidance, education still helps "some small, small amount of people" over time.

K9: One important thing that she points out is that many girls want accurate information, but they don't always know where to get it safely. Because they're afraid of being judged, they sometimes rely on friends or social media or AI, which isn't always reliable. I also think parents and communities need more education so they can support girls instead of making them feel ashamed; to create a safe space for them. 'If girls knew they could get confidential, respectful care without being judged, I think more of them would seek help when they need it.'

K10: On abstinence-only vs. comprehensive sex ed specifically, she feels like it is tied up with religion and parental authority.

K11: She said that reducing stigma, both public and internalized among women, would be the most important change to make access easier.

K12: She said there is a lot of stigma and judgment in African settings.

G. Kibera IDI Transcripts – K1–K7 (Raw Data)

This appendix reproduces the full transcripts of the seven in-depth interviews conducted in Kibera, with participant labels anonymised to K1 through K7 to match the labelling used throughout this report. "Interviewer" and "Respondent" turns are reproduced as recorded and translated by the research team.

K1 – Full IDI Transcript (Kibera)

Interviewer: Okay, uh thank you for agreeing to speak with me today. My name is Interviewer Njoroge and I'm part of research team exploring the experiences of young women in accessing sexual and reproductive health information and services here in Kibera. There are no right or wrong answers. I am interested in hearing about your own experiences and opinions. Your participation is completely voluntary. You do not have to answer any question that makes you uncomfortable and you may stop the interview at any time without any consequences. Everything you share will be kept confidential. Your name will not appear in any report and no information that could identify you will be shared. Before we begin, do you have any question about the study or interview?

Respondent: No

Interviewer: Okay, so can you tell me a little about yourself such as age, whether you are currently in school or working, and what everyday life is generally like for you here in Kibera?

Respondent: Oh, for me, I'm 19 years old currently. Uh life for me is not that bad, I'm trying to manoeuvre, trying to see what I can do, um trying to ensure that I I don't engage in things that will make me get health issues and all that. So I try to do as much as possible not to regret in any way, yeah. Like I I just prevent doing things that will make me end in hospital and all that. Yeah, and I also do projects, yeah, and all that, heh.

Interviewer: So, are you currently in school or working?

Respondent: Uh, I'm in school. Uh-huh, yeah, I'm in school. I'm not working, I used to work but I stopped, but now I'm just in school.

Interviewer: Okay, so how long have you lived here in Kibera?

Respondent: Uh, maybe let's say three months.

Interviewer: Okay, three months, okay. So, when you um when you think about uh sexual and reproductive health services in this community, for example, clinics, counselling, family planning, HIV testing, or health education sessions, what comes to your mind first?

Respondent: When I think about all that, what comes to my mind is women's health, like how they take care of themselves to ensure that they are not getting sick, because sometimes like it depends with someone. If you are not if you don't care, you'll end up in hospital. So, for me, when I when I think of all that, HIV test, everything, it's about women's health and how to take care of themselves, yeah, to avoid such things. And being clean also, yeah, it prevents you from getting sick. But also, those services um helps us to know our status and helps us also to be able to prevent and avoid some circumstances that will make us end in hospital, yeah. So they create awareness, those hospitals. When you go there and ask let's say for consultation or something,

you get like some advice on what to do, on how to prevent certain things, on how to be clean, yeah. So, when I think about all those services, I think of how women protect themselves, how they get advice, how they will be able to manoeuvre with life, and how they take care of themselves, yeah.

Interviewer: Uh, in your community, uh are young women aware of these services?

Respondent: Yeah, they are aware of them, but some people are just ignorant, but the services are made aware in social media, in billboards, all over. They are aware, in schools they are taught these things, yeah, so the awareness is there, but some people are just ignorant, yeah, of the information.

Interviewer: Uh, from your experience, what makes it difficult for girls and young women uh your age to access sexual and reproductive health information or services?

Respondent: Uh, maybe the fear of knowing their status or knowing their health. Maybe they engage in things that are not good, and they were just don't care, and they ended up maybe getting sick, but they fear the consequences or they don't want to know their status, or they fear whatever will let's say some of them their parents pressure them or something, so they fear what may come if they go and ask for consultation or something. So they that's why they are I guess it's just fear that makes them not access those um those services, yeah. Fear of the results, fear of what will happen next, yeah.

Interviewer: Okay, uh do you think some of these uh challenges make others worse?

Respondent: Yeah, some get worse and maybe they can infect others, uh they can get reinfections, they can become weak. Yeah, they can get even other diseases in addition. So, there are many consequences of not accessing those services.

Interviewer: Hmm okay. So, um when you or girls you know need information or help about uh sexual and reproductive health, where do you usually go first?

Respondent: Ah, don't pardon the question?

Interviewer: When you when you or girls uh you know needs uh information or help about sexual and reproductive health,

Respondent: Uh-huh.

Interviewer: where do you usually go first?

Respondent: Uh, mostly at the clinic or hospital. Yeah, mostly you just take yourself there. Mostly grownups, but if someone is still a child, maybe they'll have to uh get access to their parents and their parents will get them from there. But as long as one is an adult, you can just take yourself to a clinic and ask for the information you need, yeah.

Interviewer: Um, do you trust uh these sources?

Respondent: Yeah, I do, especially if it's a recognized hospital. I I do trust them because they'll give you information, at least you'll know what to do and learn a lot. Because if you don't ask them, then you will remain you'll just be there not knowing like what to do next. So I trust them, yeah, as long as the source is recognized, yeah, and the doctors are are welcoming, yeah, I trust them. But other people no, information from people no. It's better you just go to the hospital and ask for myself, yeah.

Interviewer: Um, what do you think uh might happen if people in your family or community found out that a girl was seeking sexual and reproductive health information or services?

Respondent: I don't...

Interviewer: What do you think might happen if people in your family or community uh found out that a girl was seeking sexual and reproductive health information or services?

Respondent: Um, first of all, the family will judge you. Uh, they might not exactly understand why you went and seek information. Yeah.

Interviewer: Okay so, uh have you or someone you know ever experienced difficulties after seeking sexual and reproductive health information or services?

Respondent: Um, for now no, but I'm very sure that if one tells their parents or if their parents come to know, most uh African parents will just judge you and maybe make you feel like it was not worth it. They might tell you to go back to where you came from, yeah all that.

Interviewer: So, uh how do how do uh how do this uh situation affect uh the person involved?

Respondent: Um, the person might be mentally not okay because they are not able to express themselves. Uh they might have stress because they are thinking about the matter. Maybe if they after they seek information, maybe it didn't come as they expected, so they are stressed out. It might might affect what they do, they might get depressed, others might not like their lives might not be the same, yeah. So, it affects them mentally mostly.

Interviewer: Okay, uh have you heard whether the experiences of girls in in Kibera are similar to or different from those of girls in other parts of Nairobi or elsewhere in Kenya?

Respondent: Um, they might be similar but not very similar. Some even in other parts of Nairobi experience the same thing, yeah, as those in Kibera. So, they are sometimes similar, yeah.

Interviewer: Uh, why do you think uh those differences exist or why are they similar?

Respondent: Um, I said they were they are a bit similar because those in Kibera and those in other parts of the of Nairobi they all um seek the same information from different sources and the information might affect them. Yeah, so despite not being in Kibera does not mean that you won't be affected as those in Kibera. And the differences come in um maybe in terms of the way of living, how you are you are brought up might bring the difference, but yeah, that's the difference. But similarity is both of them will be affected mentally, yeah.

Interviewer: Okay, so what changes do you think uh would make it easier for girls and young women in Kibera to access sexual and reproductive health information and services?

Respondent: Uh, maybe building more clinics there, ensuring that there's cleanliness. Because you know you can't have a hospital near places that are very dirty. They need to say like clean the environment and set up a clinic near there that's affordable for those girls to access and ask for information. Yeah.

Interviewer: Okay, so is there anything else about this topic that you think is important for us to understand that we have not discussed?

Respondent: Okay um when like when someone let's say seeks information, the people who are giving information should not judge this person. Because some people don't seek for information because let's say they don't want, it's because let's say maybe this doctor will start judging you or will maybe harass you. So, what can I can say needs to change is how doctors see these girls, yeah, even if they are underage, they need to understand them and listen to them. Because some doctors from the way I've seen some of them, they don't really listen to someone, they start judging you because you are underage and something. So, I guess that part. Yeah, doctors not judging people. Not all but some do, yeah.

Interviewer: Okay, uh thank you very much for taking time to share your experience and perspective today. Your contribution is very valuable and will help us uh better understand the challenges young women face in accessing sexual and reproductive health information and services. Once again, thank you.

K2 – Full IDI Transcript (Kibera)

Interviewer: Okay, thank you for agreeing to speak with me today. My name is Interviewer Njoroge, and I'm part of a research team exploring the experiences of young women in accessing sexual and reproductive health information and services here in Kibera. There are no right or wrong answers. I am interested in hearing about your own experiences and opinions. Your participation is completely voluntary. You do not have to answer any question that makes you uncomfortable, and you may stop the interview at any time without any consequences. Everything you share will be kept confidential. Your name will not appear in any report, and no information that could identify you will be shared. Before we begin, do you have any question about the study or the interview?

Respondent: No, no, no. We can start.

Interviewer: Okay. So, can you tell me a little about yourself, such as your age, whether you are currently in school or working, and what everyday life is generally like for you here in Kibera?

Respondent: Um, my name is Respondent. I am uh, I'm 19 years old, um, and I'm a student. Uh, first year and um, everyday life just looks like, um, you know, just studying and normal things, going to classes, come back, rest. Uh, yeah, just basically that, unless I've-I've gone home for the holiday.

Interviewer: And how long have you lived here?

Respondent: Uh, I've been living here for about uh, around, let's say four-four years, four years.

Interviewer: Four years, okay. So, when you think about uh, sexual and reproductive health services in this community, for example, clinics, counselling, family planning, HIV testing, or education sessions, what comes to mind first?

Respondent: I think maybe what would come to mind would be um, um, I don't think we-we have much of that because, uh, it's-it's more of something that is shunned in the-in the, in this place. So, I'd say the farthest we've come maybe would be maybe in, um, what are they called? Pharmacies, where you can go and buy products and whatnot. But, in terms of counselling, in terms of testing, in terms of, you know, knowledge about family planning and all those things, I'd say we are a bit uh, far off. And there isn't much exposure to that, yeah.

Interviewer: Okay. So, from your experience, what make it, uh, what makes it uh, difficult for girls and young women your age to access uh, sexual and reproductive health information or services?

Respondent: First of all, I-I'd say it's the notion that, as-as a woman... You see, for-for men it's different because uh, when you go to-to access such services, people will be like, "Yeah, it's-it's common." But if-if-if for a woman, they can take you for, you know, a prostitute or-or let's say, the term "slut," because they wonder, "Why are you looking for these services?" So, it's more of um, a stereotype. It looks wrong for someone to be looking for that because it makes you look like you are morally, like you are of low moral standing. So, that's the first thing I'd say. And also, standard, the second thing is that I think most-most women, uh, the-the environment around us isn't really uh, building us to be sexually positive because there are-there are so many things that stand in our way. For example, you'd hear, "Oh, you might..." like, you know that-that um, I don't know how to explain it well, but it's more of those-those stereotypes, and the way, the view people have of you-have of you when you-when you try seeking such services. It seems like, yeah, like I said, a slut or, let's say, a prostitute. Yeah.

Interviewer: Uh, okay. So, uh, do you think some of these uh, challenges makes, uh, make others worse? Maybe, for instance, does uh, maybe fear of being judged uh, prevents girls from visiting clinics even when services are available?

Respondent: Yeah, that does, by far. Because, you know, uh, we are-we are, we're living in a time where like, from-from what I've-I've seen from other people, it's like even the people in those clinics, they're also very judgmental. Like, for example, there's this one case where uh, a girl was going to-to get tested, a pregnancy test. And then uh, the nurse was like, "Eeh, ata wewe?" (Oh, even you?) So, you see that-that-that-kind of-that kind of output, it makes you even, it makes you feel like, like you begin asking yourself, "What's the point of even going there because you know you're going to be judged?" And, and even the people there... Cuz I believe as a female, as I go there, I expect to meet another female. So, the idea of another, you meet there a male who's also going to maybe try to throw passes at you... So, I feel like, though yeah, you know we should be aware that even men can do that job, but of course for comfortability, I'll be more comfortable if I meet another lady there, who we can easily-we can easily be on the same ground, we can easily like, how do you call it? You can easily understand each other. Yeah, because we are both in the same, like in the same gender category. So, yeah, that's what I think.

Interviewer: Okay. So, uh, when you need information or help about uh, sexual and reproductive health, where do you usually go first?

Respondent: Mostly, I go to Google. Like, I just-I just search online, mm-hmm, uh, because online, it's-it's my phone, it's my own. There is, there is some level of privacy. Cuz Google isn't a human; it wouldn't judge me. It wouldn't really, um, you know, ask me, "Why are you asking about this?" So, mostly I'd go online. And if-if not online, I'd maybe talk to a trusted friend who I know has actually gone through this, has experience in it. But aside from that, those are the

only, you know, the places where I feel like my privacy is-is well-well kept, like, like I don't have to worry about it. Because at-at-at the end of the day, I can delete my search history. And of course, I know um, there are-there are ways in which someone can-can like, retrieve it, but of course, that isn't really what-what um, matters at that point in time. Yeah.

Interviewer: Okay. So, uh, what do you think uh, might happen if people uh, in your family or community found out that a girl was seeking sexual and reproductive health information or services?

Respondent: So, first of all, I'd say I'd-I'd assume, or okay, from what I've-I've-I've experienced, they'd be thinking... Because you know, there's this whole um, from the older ages, let me just say the-the millennials or the, our parents' ages and our grandparents' ages, to them, the-the notion of sex before marriage is very much shunned. So, they'll be wondering, "Why do you need this information yet you aren't married?" But of course, we need information whether or not you're getting married because you never know. So, that's the whole-the whole idea is first of all, I'm sure there'd be judgmental, there'll be an understanding, there'll, they'll of course, misunderstand. They'll assume you are-you are, you are on the road to being sexually active, which to them is immoral because it isn't in the confines of marriage. So, I think they'd be very, very judgmental and they'd most likely shun you, or the, what do you call it?, they'd isolate you. Because, for example, they'd be like... For example, let's say people in the neighbourhood, they'll be like, they'll be telling their children, "Don't be like that girl." You see, such things. So, you'd be shunned, isolated, judged, misunderstood, among other things. Yeah.

Interviewer: Okay. So, uh, have you or uh, someone you know ever experienced uh, difficulties after seeking sexual and reproductive health information or services?

Respondent: Um, none, none. Most of the ones like, I-I for one went to take uh, an HIV test, just cuz-cuz I wanted to know my status. And, and I also invited a friend of mine so that we could go

take it, and she was-she was very receptive. Because it depends where you are taking it from and, of course, the-the centre you are accessing it from. Because some centers are very high on your privacy, on your data. So, so far, I think-I think I haven't-I haven't experienced any problem, neither do I know anyone who has experienced any problem. Mostly because of the center, the place where you go to get it. If you get it locally, most likely that nurse is your neighbor, and they're going to say, "Oh, you know, this girl, I saw them. Oh, they-they came to the office yesterday," and such. So, mostly, I think taking it where you are sure your privacy is going to be upheld. That's what I've uh, noticed from most of the people around me who sought such services.

Interviewer: Mm, okay. So, uh, have you heard uh, whether the experiences of girls uh, in Kibera are similar to or different from those-those of girls in other parts of Nairobi or elsewhere in Kenya?

Respondent: I feel like it's very, very different because people from Kibera, um, I can say, there's first of all, there's less awareness. Secondly, there's-there's, of course, access to such services is very minimal. So, compared to someone who lives somewhere in let's say, Kitengela, or other places of Nairobi, even in Nakuru, those people they-they-they have, first of all, there's more awareness. People know these things are-are, you know, available. And of course, there-there is of course, let's say, for example, in Kibera the-the access to internet is very, very minimal, and you know, nowadays, most information flows through the internet. Like, as of now there's, um, there's a page on Shujaz, on-on Instagram called Shujaz where they actually, they talk a lot about it. So, the whole idea is even the-the information isn't there for people in Kibera unless you actively seek it out. So, yeah, that's what I'd say. It's very different in terms of access to-to services and information, and also the amount of stereotypes that fall in those specific, because in Kibera, there's-there's a lot of-a lot of stereotypes compared to other places where there's more information breaking those stereotypes. Yeah.

Interviewer: Okay. So, uh, what changes do you think will make it easier for girls and young women in Kibera uh, to access sexual and reproductive health information and services?

Respondent: So, first of all, providing awareness in schools, the schools where these girls are going to. Providing awareness there, and also providing awareness in the community at large, having seminars, open-air seminars where we-we are able to educate the parents that these things are not things you should shun your kids for trying to-to reach out to. So, first of all, awareness is very, very important. And also, uh, provision of information, having pamphlets, having, you know, access to internet, having, um, books in schools, having even guidance and counsellors in schools and even having offices for them within the locality of Kibera so that you can just go and talk to them. That's very important. Also, providing those centers and those places where you can easily seek out those um, services, and at the same time, ensuring that those people are qualified and they are qualified to a sense that they-they're-they're also aware of the privacy of the patient, of the-of the client they're meeting. So, that's very, very important. And yeah, that's basically what I think uh, would be at the top of my list of priorities of things that uh, would really help the ladies and females in this place, yeah.

Interviewer: Okay. So, is there anything else uh, about this topic that you think is important for us uh, to understand that we have not discussed?

Respondent: Not really, I feel like the questions have greatly touched on-on all the possible spheres. Yeah.

Interviewer: Ah, okay. Uh, thank you very much for taking the time to share your experiences and perspectives today. Your contribution is very valuable and will help us uh, better understand the challenges young women face in accessing SRHR services. So, some of the topics uh, we discussed can be personal or emotional. If this conversation has raised uh, any

concerns or difficult feelings, I can provide you with uh, contact information where you can receive confidential support if you wish. And, thanks again.

K3 – Full IDI Transcript (Kibera)

Interviewer: Okay, uh, thank you for agreeing to speak with me today. My name is Denis Njoroge and I'm a part of a research team exploring the experiences of young women in accessing sexual and reproductive health information and services here in Kibera. There are no right and or wrong answers. I'm interested in hearing about your own experiences and opinions. Your participation your participation is completely voluntary. You do not have to answer any question that makes you uncomfortable, and you may stop the interview at any time without a without any consequences. Everything you share will be kept uh, confidential. Your name will not appear in any report and no information that could identify you will be shared. Before we begin, do you have any question about the study or the interview?

Respondent: No, I have no. I think we can just get started.

Interviewer: Yeah. Okay, let's start. Uh, can you tell me a little uh, a little about yourself such as age, whether you are currently in school or working, and what everyday life is generally like for you here in Kibera?

Respondent: Uh-huh. I am 19 years old. I just finished my first year in school. I am partially working as well with, uh, a bit of gig work here and there. My life here in Kibera, I'd have to say, is an experience everyday, like, there are no days the same as the next.

Interviewer: Okay, so how long have you lived here?

Respondent: Four years.

Interviewer: Four years, okay. So, uh, when you think about uh, sexual and reproductive health services in this community, for example, clinics, counselling, family planning, HIV testing, or health education sessions, what comes to your mind first?

Respondent: What comes to my mind first is the counselling that they provide to girls, mostly in school, and how important it is and how it shapes them to be more knowledgeable in the community.

Interviewer: Mhm. So, uh, are young women uh, aware of these services?

Respondent: I'd say um, I'd say they're not as aware, or uh, there's still a huge percentage that is not aware of the services that are going on. But you may go out and find yourself um, just interaction here and there make you realize that some people are not even aware that these services exist, or things that you have to pay such large amounts for you to acquire these services when it should be rather easily accessible to them because they need it most.

Interviewer: Okay, uh, from your experience, what makes it difficult for girls and young women your age to access sexual and reproductive health information or services?

Respondent: Um, um, at the top of my head, I'd say there are some factors you can see. The first one would also be uh, peer influence. By that I mean, you may find that uh, for example, I have a friend. A friend will, I'm having some issues and I think something is wrong. I open up to that friend and I'm like, "Hey, this and this is going on." But that friend will be like, "Oh, real?" or they give some advice that really isn't fit, or doesn't recommend or point to you going and seeking the medical advice uh, medical instance. Another thing I'd say is um, even though we are developed in this era, there's still so many people who don't have access to social media, or whenever they are on social media, there are not as many platforms to interact or to raise awareness of women's health and women's um, sexual sexual hygiene.

Interviewer: Mhm, okay. So, uh, do you think some of these uh, challenges makes others worse? Or okay, for instance, uh, does uh, fear of being maybe judged prevent girls from visiting clinics even when services are available?

Respondent: I can say it does contribute a lot because once that fear is instilled of what are people going to think about me, oh, what if I go and find out that this is much more serious than I already think it is, what's going to happen, or my parents going to perceive it? Once they already have that fear instilled in them, they'd rather, even if the services are there, those few services that are present, they um, the people will not go to seek them because of, again, that deeply instilled fear from the community around them.

Interviewer: Okay, so, uh, when you or girls you know need uh, information or help about sexual and reproductive health, where do you usually go first?

Respondent: Um, for me personally, I have gone through, I've made it a point to go over some of the books that are available online from validated sources on that can help on identifying or me learning more about my reproductive health, uh, which I have seen help a lot, but also access to, make it a point to have a checkup every now and then.

Interviewer: Uh, do you trust those sources?

Respondent: I [laughter], I can say yes, they're very validated because they're medical research um, findings and books used to uh, learn about the female reproductive system.

Interviewer: Ah, okay. So, uh, what do you think uh, might happen if people in your family or community found out that a girl was seeking sexual and reproductive health information or services?

Respondent: Sorry?

Interviewer: Uh, what do you think might happen if people in your family or community found out that a girl was seeking sexual and reproductive health information or services?

Respondent: Um, I think I'd see for, I'm sorry, I can't speak for the same, but for my family, they're quite open-minded because my mother was actually the one who's pointed me to such resources. But when you go to, let's say rather more of the extended family, you see that there's a tendency to believe that a girl who's teaching about uh, the sexual health or um, reproductive health to other other people, there's kind of this backward belief that you have to have gone through this and this, or you have to be this kind of a person, that's why you've resulted into it. But um, really uh, my family wouldn't have a specific loss.

Interviewer: Mhm, okay. So, uh, have you or someone you know ever experienced difficulties after seeking sexual and reproductive health information or services?

Respondent: Um, hm. You ask if if me or someone related to me had, pardon, the past?

Interviewer: Okay, uh, have you or someone you know ever experienced difficulties after seeking sexual and reproductive health information or services?

Respondent: Oh, yes, yes, yes, yes. Um, there was a classmate of mine who got um, yeah, one of my classmates got knocked up and in her attempt to, you know, uh, get rid or if, as they say, to wash, uh, to get an abortion, she resorted out to some medical fields that were not really certified, which led to a lot of complications cuz there was foaming of the mouth, uh, they had to, she was bleeding profusely, there were just so many things that happened in to a point where you realize that this um, particularly uh, things like abortion pills are not regularly available to girls who need it most, but that's still not as open to the community as we may think.

Interviewer: Okay, so, uh, have you heard um, whether the experiences of girls in Kibera are similar to or different from those girls in other parts of Nairobi or elsewhere in Kenya?

Respondent: I'd say they are, the cases in Kibera are reflective of what's going on in the whole country in general because um, I'd say what the what the things that are happening is they don't have, there are still clinics, some clinics are opening up here and there, but there's not as much information going out to the girls in Kibera about their reproductive health, how they can take care of themselves, what to do and where to go in case something happens. So, yeah.

Interviewer: Um, okay. So, uh, what changes do you think uh, would make it easier for girls and young women in Kibera to access sexual and reproductive health information and services?

Respondent: I'd say for more organization to actually go down to the deep roots of the of Kibera itself and talk to girls, you know, and not just the girls. The girls can, the girls, the young women, their mothers as well, just more civic education on how and why it's important to have these services and just raise general awareness to de- de-color the idea that sexual health is not as important as we think because it is very important.

Interviewer: Okay, so is there anything else about this topic that you think is important for us to understand that we have not discussed?

Respondent: Um, I think we'd have, but one thing I'd like to bring to mind is the medical, as much as we say the medical services, there are not as many gynaecologists who are available to those who need it the most, or uh, you may find that some would want to seek that help but they find it too expensive. So, I'd say that is something that will need to be brought to light.

Interviewer: Okay. Uh, thank you very much for taking the time to share your experiences and perspective today. Your contribution is very valuable and will help us to better understand the challenges young women face in accessing sexual and reproductive health information services. Uh, thank you again for your time.

K4 – Full IDI Transcript (Kibera)

Interviewer: Okay, thank you for agreeing to speak with me today. My name is Denis Njoroge and I'm part of a research team exploring the experience of young women in accessing sexual and reproductive health information and services here in Kibera. There are no right or wrong answers. I am interested in hearing about your own experiences and opinions. Your participation is completely voluntary. You do not have to answer any question that makes you uncomfortable, and you may stop the interview at any time without any consequences. Everything you share will be kept confidential. Your name will not appear in any report, and no information that could identify you will be shared. Before we begin, do you have any questions about the study or the interview?

Respondent: No.

Interviewer: Okay, so we begin. Can you tell me a little about yourself, such as your age, whether you are currently in school or working, and what everyday life is generally like for you here in Kibera?

Respondent: My... my name?

Interviewer: No, start with your age.

Respondent: Oh. I am... I am 19 years old. I am studying in University of Nairobi.

Interviewer: Okay. How's your life in Kibera?

Respondent: My life is good.

Interviewer: Okay. So how long have you lived here in Kibera?

Respondent: Four years.

Interviewer: Four years, okay. When you think about sexual and reproductive health services in this community, for example, clinics, counseling, family planning, HIV testing, or health education sessions, what comes to your mind first?

Respondent: Repeat the question.

Interviewer: Okay. When you think about sexual and reproductive health services in this community, for example, clinics, counseling, family planning, HIV testing, or health education sessions, what comes to your mind first?

Respondent: What comes to my mind... like in terms of clinics, the treatment? What should I... in terms of?

Interviewer: So, in terms of what kind of services are available, and how aware are young women of these services?

Respondent: The awareness is ritual because not... because few people are educated. And the services, they are poor because the clinics are ritual and there are few medics.

Interviewer: Okay. So from your experience, what makes it difficult for girls and young women your age to access sexual and reproductive health information or services?

Respondent: Maybe their... their poor background. And... maybe the lack of knowledge, the ignorance.

Interviewer: So do you think some of these challenges make others worse?

Respondent: Make others worse? Like other people?

Interviewer: For example, does a fear of being judged prevent girls from visiting clinics, even when services are available?

Respondent: Yeah. They fear... maybe they fear discrimination from others. So they don't want their status to be known.

Interviewer: Okay. When you or girls you know need information or help about sexual and reproductive health, where do you usually go first?

Respondent: So, where do I go first? Like how I advise them?

Interviewer: Like, if you need help about sexual and reproductive health, who is the first person to approach?

Respondent: Oh, like me?

Interviewer: You or any other person.

Respondent: Oh. Maybe my peers.

Interviewer: Your peers, okay. So do you trust this source?

Respondent: Yes.

Interviewer: Why do you trust them?

Respondent: Because... we are the same age, like we won't judge each other. We are open to each other. So we will share everything.

Interviewer: Okay. What do you think might happen if people in your family or community found out that a girl was seeking sexual and reproductive health information or services?

Respondent: How the community will react?

Interviewer: Yeah, or your family.

Respondent: Maybe they will think you are... they will take it negatively. Like... unaweza explain na Kiswahili, hakuna shida (you can explain in Swahili, no problem). Sasa, nasema maybe watadhani uko na tabia mbaya (Now, I'm saying maybe they will think you have bad behavior).

Interviewer: Okay, so have you or someone you know ever experienced difficulties after seeking sexual and reproductive health information or services?

Respondent: Like, difficulties in...?

Interviewer: Difficulties after seeking sexual and reproductive health information or services.

Respondent: From anyone? Like difficulties like kupata mimba (getting pregnant) or what difficulties?

Interviewer: For example, you've gone to seek sexual and reproductive health information or services, what happened to that person, or how did the situation affect the person involved?

Respondent: How did the situation after going to seek the information affect the person? They are able to decide what to do, like how to deal with... they are able to decide about their sexual life. They become knowledgeable.

Interviewer: Okay, so we can put it like that. Have you heard whether the experiences of girls in Kibera are similar to or different from those of other girls in other parts of Nairobi or elsewhere in Kenya?

Respondent: They are similar.

Interviewer: So, what kind of similarities exist?

Respondent: The fear of the community, and lack of enough knowledge about their sexual health.

Interviewer: Okay. What changes do you think will make it easier for girls and young women in Kibera to access sexual and reproductive health information and services?

Respondent: Maybe organizing seminars to educate young girls about their sexual life.

Interviewer: Is there anything else about this topic that you think is important for us to understand that we have not discussed?

Respondent: I don't think so. No.

Interviewer: Okay, thank you very much for taking the time to share your experience and perspective today. Your contribution is very valuable and will help us understand the challenges young women face in accessing sexual and reproductive health information and services. Once again, thank you.

K5 – Full IDI Transcript (Kibera)

Interviewer: Okay, thank you for agreeing to speak with me today. My name is Denis Njoroge and I'm part of a research team exploring the experience of young women in accessing sexual and reproductive health information and services here in Kibera. There are no right or wrong answers. I am interested in hearing about your own experiences and opinions. Your participation is completely voluntary. You do not have to answer any question that makes you uncomfortable, and you may stop the interview at any time without any consequences. Everything you share will be kept confidential. Your name will not appear in any report, and no information that could identify you will be shared. Before we begin, do you have any questions about the study or the interview?

Respondent: No.

Interviewer: Can you tell me a little about yourself, such as your age, whether you are currently in school or working, and what everyday life is generally like for you here in Kibera?

Respondent: I am 21 years old. Right now, I am not in school because of fees, but I do some small business selling second-hand clothes near Olympic stage when I get some capital, or I help out at a friend's salon. Life here is just about everyday hustling. You wake up, look for water, and see what you can earn for the day. It's vibrant; everyone knows everyone. However, it can be tough because money is always tight and there is very little privacy.

Interviewer: How long have you lived here?

Respondent: I was born here. I've lived in Kibera area my whole life.

Interviewer: When you think about sexual and reproductive health services in this community, for example, clinics, counselling, family planning, HIV testing, or health education sessions, what comes to mind first?

Respondent: Honestly, the first thing that comes to mind is judgment. If you walk into a clinic as a young, unmarried girl, people immediately start looking at you sideways. But it also means protection and trying to avoid mistakes like getting pregnant before you are ready to feed a child.

Interviewer: What kinds of services are available? How aware are young women of these services?

Respondent: Services are available. We have places like AMREF, MSF (Doctors Without Borders), and local public dispensaries. There are also chemists everywhere. Girls my age know about family planning, like the emergency pill (P2), implants, and condoms, and we know where HIV testing is done. The awareness is high because of NGOs, but getting access to them is the hard part.

Interviewer: From your experience, what makes it difficult for girls and young women your age to access sexual and reproductive health information or services?

Respondent: The biggest challenge is privacy and confidentiality. Kibera is very crowded. If I go to the local clinic, the woman at the reception might be my mother's friend or a neighbour from our court. By evening, the whole neighbourhood will know you went to ask for family planning. Another issue is the attitude of healthcare workers, especially in government clinics. They will lecture you. They ask, 'You are so young; why are you doing these things? Where is your husband?' It makes you feel ashamed. Also, cost is a factor. Even if the service is free, you might need 50 shillings for a card, or money for transport if you decide to go to a clinic far away where people don't know you.

Interviewer: Do you think some of these challenges make others worse? Can you give an example?

Respondent: Yes, they definitely connect. For example, because of the fear of being judged by local nurses (lack of privacy/attitude), a girl may choose to go to a private chemist instead of a free clinic. But at the chemist, she has to pay cash for pills or an injection. If she doesn't have that money because she is unemployed (cost), she ends up doing nothing. She takes the risk, and that's how unwanted pregnancies happen.

Interviewer: When you or girls you know need information or help about sexual and reproductive health, where do you usually go first?

Respondent: First is always friends or peers. We talk among ourselves a lot. If that fails, the internet or social media, TikTok and Google have everything nowadays. You just search anonymously. You rarely see a girl going to her parents or a church leader. That is like looking for trouble.

Interviewer: Which of these sources do you trust the most? Why do you trust them?

Respondent: I trust the internet for facts, but I trust my close friends for real-life experience. If a friend tells me, 'Go to this specific clinic, there is a nice nurse who won't yell at you,' I will trust her completely. We don't trust the older generation or local gossip groups because they use that information to ruin your reputation.

Interviewer: What do you think might happen if people in your family or community found out that a girl was seeking sexual and reproductive health information or services?

Respondent: Eh, the gossip will be too much. In the family, you can even be beaten or chased away because they think you have become loose or a prostitute. They will say you are bringing shame to the family. In the community, people will start pointing fingers at you, and if you are looking for a serious relationship later, it becomes hard because they've already labelled you.

Interviewer: Have you, or someone you know, ever experienced difficulties after seeking sexual and reproductive health information or services? (Please only share what you are comfortable discussing.)

Respondent: I have a friend who got pregnant last year. Before that, she went to a local facility to ask for an implant. The nurse told her she was too young and that the side effects would make her barren in the future, which wasn't true. My friend got scared and left without protection. Two months later, she was pregnant. She had to drop out of her catering course. It changed her whole life just because one nurse gave her wrong information out of malice.

Interviewer: Have you heard whether the experiences of girls in Kibera are similar to or different from those of girls in other parts of Nairobi or elsewhere in Kenya?

Respondent: I think for girls in places like Westlands or Kilimani, it's different. They have money to go to private hospitals where nobody cares about your life. You just pay, get your

service, and leave. Here in Kibera, poverty makes you exposed. You rely on public spaces where you have zero privacy. But the fear of parents finding out? I think that is the same for all Kenyan girls.

Interviewer: What changes do you think would make it easier for girls and young women in Kibera to access sexual and reproductive health information and services?

Respondent: We need youth-friendly clinics that are completely separate from the main maternal health clinics. The staff there should be young people who understand us, not older women who want to preach to us. Also, digital services would help, like a system where you can chat with a doctor on WhatsApp, ask all your questions, and just go to the clinic to pick up what you need quickly without queuing or talking to ten people.

Interviewer: Is there anything else about this topic that you think is important for us to understand that we have not discussed?

Respondent: Just that people shouldn't look at young women in Kibera as if they are reckless. Girls here want to achieve their goals, go to school, and work. They seek these services because they are trying to be responsible for their futures. If the community stops treating contraception like a sin, fewer girls will suffer.

Interviewer: Okay, thank you very much for taking the time to share your experience and perspective today. Your contribution is very valuable and will help us understand the challenges young women face in accessing sexual and reproductive health information and services. Once again, thank you.

K6 – Full IDI Transcript (Kibera)

Interviewer: Okay, thank you for agreeing to speak with me today. My name is Denis Njoroge and I'm part of a research team exploring the experience of young women in accessing sexual

and reproductive health information and services here in Kibera. There are no right or wrong answers. I am interested in hearing about your own experiences and opinions. Your participation is completely voluntary. You do not have to answer any question that makes you uncomfortable, and you may stop the interview at any time without any consequences. Everything you share will be kept confidential. Your name will not appear in any report, and no information that could identify you will be shared. Before we begin, do you have any questions about the study or the interview?

Respondent: No.

Interviewer: Can you tell me a little about yourself, such as your age, whether you are currently in school or working, and what everyday life is generally like for you here in Kibera?

Respondent: I'm 19 years old a student here in Kibera. Life in Kibera generally is fair. I can say it's fair, it's not hard, it's not easy. You do have to put in some work and effort, especially when it comes to... yeah, I can say it's not difficult, it's just fair. It's in between; it's neither harsh nor tough.

Interviewer: When you think about sexual and reproductive health services in this community, for example, clinics, counselling, family planning, HIV testing, or health education sessions, what comes to mind first?

Respondent: What comes to mind first when I think about sexual and reproductive health services... um, I'm not generally sure, but all I can say is that it's helpful. It is really helpful, especially for the youth. Yeah, when it comes to dealing with sexual and reproductive health, because I feel that they should provide a safe place for many youths in Kenya. This is especially true because of strict parents; if you find yourself in a very tough situation, it's hard to run to your parents, most parents, you know, it's hard to run to your parents. So, I do believe that they

are essential in the community because their main functionality, I think, is to provide a safe place for everyone.

Interviewer: From your experience, what makes it difficult for girls and young women your age to access sexual and reproductive health information or services?

Respondent: One barrier we can talk about is fear, maybe fear of being seen. You know, when you want to go somewhere and you're like, "How will the doctor take me?" or "How will they..." You know, there are so many questions, but the main one is fear. Fear of judgment, because you don't want to be judged. Number two, maybe their beliefs. There is this belief that people, actually, young women and men, don't go to hospitals. They prefer giving birth at home because maybe of witch doctors or something like that. But, yeah.

Interviewer: Do you think some of these challenges make others worse? Can you give an example?

Respondent: One of such difficulties is when you have the the premonition of you're not supposed to go to these services, maybe you're supposed to sit down, stay at home, gets give birth using midwives like in the traditional ways. So you know that instils fear, that instils myths into one's head, you're like, "Ah, if I go to the hospital, one, two, three happens." Yeah.

Interviewer: When you or girls you know need information or help about sexual and reproductive health, where do you usually go first?

Respondent: Um, today we do seek help from social medias, from your trusted friends. Yes.

Interviewer: Which of these sources do you trust the most? Why do you trust them?

Respondent: Um, social media because there is a lot of information on social media, but the most difficult thing is to differentiate between what's true and what's false. But if you do your

research properly, you're able to identify, critically discern, the truth and the fault false information because where there is truth, there is always going to be a bit of false, and if false information is there, a bit of truth is always going to be there. Friends, um, I have friends like maybe who are doctors, who are nurses, so it is good to also counter-check by them. That means you read information from social media and your friends are like mature, um, they have experienced life. I do believe that they have experienced life better than I have. So it's good to like run uh by them uh about the few facts that you find on social media.

Interviewer: What do you think might happen if people in your family or community found out that a girl was seeking sexual and reproductive health information or services?

Respondent: Um, personally, my family wouldn't have like any issues if I was to be able to seek SRHR services. ...um SRHR services or information. Um information, I I don't think that they would have like any problem with one seeking information, but the services will depend, um especially if you're looking for um especially there would be a lot of judgment if you're looking for like family planning as a young girl because they do believe that the girl should shouldn't be like so much involved with the sexual and uh you know sexual things. So they do prioritize a girl maybe gaining education first, maybe getting a work, that good job, that degree first before engaging in other other matters. So I do feel like it will depend on the services that uh the girl is going to look for. But generally um not bad. Mhm

Interviewer: Have you, or someone you know, ever experienced difficulties after seeking sexual and reproductive health information or services? (Please only share what you are comfortable discussing.)

Respondent: Uh, so far I don't know anyone who has gone to seek such kind of services and, uh, things have gone west or south.

Interviewer: Have you heard whether the experiences of girls in Kibera are similar to or different from those of girls in other parts of Nairobi or elsewhere in Kenya?

Respondent: Um, uh, what I'm going to say, I prefer, I have heard some of the stories, but I can't be certain because, you know, they're hearsay. So, I won't give much of the details because they're hearsays and they're not facts and I'm not sure about it.

Interviewer: What changes do you think would make it easier for girls and young women in Kibera to access sexual and reproductive health information and services?

Respondent: Establishing separate, youth-friendly wings in existing clinics or standalone youth centers where young women can seek services privately without interacting with older community members. Training and equipping young women within Kibera to act as peer educators who can distribute information, distribute products like sanitary towels, and guide their peers to safe clinics.

Interviewer: Is there anything else about this topic that you think is important for us to understand that we have not discussed?

Respondent: Uh, I don't think there's anything important for you guys to know that you have not already discussed.

Interviewer: Okay, thank you very much for taking the time to share your experience and perspective today. Your contribution is very valuable and will help us understand the challenges young women face in accessing sexual and reproductive health information and services. Once again, thank you.

K7 – Full IDI Transcript (Kibera)

Interviewer: Okay. Uh, thank you for agreeing to speak with me today. Um, my name is Denis Njoroge and I'm part of a research team exploring the experiences of young women in accessing sexual and reproductive health information and services here in Kibera. There are no right or wrong answers. I'm interested in hearing about um your own experiences and opinions. Your participation is completely voluntary. You do not have to answer any question that makes you uncomfortable, and you may stop the interview at any time without consequences. Everything you share will be kept confidential, your name will not appear in any report, and no information that could identify you will be shared. Before we begin, do you have any question about the study or the interview?

Respondent: No.

Interviewer: Okay, so the first question is: can you tell me a little about yourself, such as your age, whether you are currently in school or working, and what everyday life is generally like for you here in Kibera?

Respondent: Okay. [clears throat] I'm I'm 19 years old. Yeah, I'm in school. I'm learning.

Interviewer: So what uh what uh everyday life is generally like for you here in Kibera?

Respondent: What everyday life is like... Okay. Okay, let me... Um-hum. Okay, so okay, as for me, okay, as for me what I can say is that okay, for me everyday life like, for me it's just simple. Yeah, I'm not working, I'm not doing any any anything. Like, I'm just at home doing some social media stuff, yeah. Like exploring and whatever, yeah. Just at home chilling, like most of the time I rarely go outside. Like, I rarely interact with people outside since since I left school. Like, I'm just at home, yeah. So let's say like my life is my life at the moment is revolving around the house. Like, sijapata kutoka nje for for some good days, yeah. So I'm just at home, yeah.

Interviewer: Ah, okay. So, uh how long have you lived here in Kibera?

Respondent: Uh-huh. [clears throat] Okay. Uh, let's say I've lived [laughs] for like one year now, yeah.

Interviewer: Okay, one year. Okay. So, um when you think about uh sexual and reproductive health services in this community, for example, clinic, counselling, family planning, HIV testing, or health education sessions, what comes to your mind first?

Respondent: Okay, so for all those things, all those clinics, everything, what comes into my mind is that people have like people are not okay, like [laughs] people have issues. Not like as a as a as a as a as a as people living in Kibera, like yeah, there are, let's say diseases and everything, yeah. And all those. So, those healthcare's they're there to receive the patients, yeah. For them to be guided, counselling, yeah. Like on those family planning, yeah. There are contraceptives, people use them as ways to prevent what? Pregnancy, or what? Um. They contraceptive methods, I think.

Interviewer: Oh, so um uh from your experience, uh what makes it difficult for girls and young women your age to access sexual and reproductive health information or services?

Respondent: What makes them? What makes it difficult?

Interviewer: Yes.

Respondent: Okay. So, for me I think they are they are they are afraid of humiliation kind of, I don't know. But they are they are just afraid of knowing their status. Something like that, yeah. Like someone is like, "Yeah, I'm young at this age, like how do I start going to for for counselling and all those?" Like, yeah. So, for them they sound kind of kind of kind of scared for them. Um-hum.

Interviewer: Mm, okay.

Respondent: So, they find it difficult to go and visit those facilities.

Interviewer: Uh okay. So, uh do you think uh some of these challenges make others worse?

Respondent: Nope. No.

Interviewer: No. Uh can you give maybe an example, maybe?

Respondent: Okay. So, these challenges that that the let's say the youth that are going through, it's not like they're making nini other challenges more difficult, yeah. 'Cause they can be worked on.

Interviewer: Um-hum.

Respondent: Yeah. Like, what if what if this problem is worked upon, yeah? So, the other problem it will be it it will be easier to work for it at the same time.

Interviewer: Okay. So um when uh when you or girls you know uh need information or help about um sexual and reproductive health, where do you usually go first?

Respondent: Hello?

Interviewer: Oh, so the next question is...

Respondent: Uh-huh.

Interviewer: ...uh when you or girls you know need information or help about sexual and reproductive health, where do you usually go first?

Respondent: For the for the girls only?

Interviewer: Or even young women, yeah. Yeah.

Respondent: Okay. So, I think they we go we visit the chemist, yeah. 'Cause we find them easier to deal with, yeah. With this coming generation like, okay. We are we are also afraid of visiting the hospitals 'cause we feel like we're gonna be asked for more money, yeah. So, also people are afraid of being to be told the full reality there. Okay, so people are going to consult first in the chemist like, "Yeah, I have this issue and this and this," yeah. So that they have a clue like what is wrong with them. So, for them they usually run to these small clinics, to the chemist, yeah. So, they explain their conditions to the pharmacist, yeah. So, they once they realize what's wrong, that's when maybe they take action to visit the hospitals.

Interviewer: Uh, do you trust the source?

Respondent: Yeah.

Interviewer: Uh, why? Why do you trust it?

Respondent: Um-hum. 'Cause, okay, from for me like I feel like, yeah, they are they are learned people. For someone to open up like something like a pharmacy, like that's someone who went through school and learnt on that course, on that particular course. Like, he won't be selling drugs... Yeah, I know they are scammers too, but for my experience, like maybe if I consult about something, I'm always given full information. Like, yeah, I'm told this and this. So, I've never questioned like this person no, this person is not learned or what. So, yeah. So from their knowledge that they learnt, that's when they came to open the chemist to sell the drugs. So, when you go there, you receive drug for your disease you're suffering from, yeah. Let's say you've been having rashes and whatever, yeah. So, they'll try telling you that they are caused by this and this and this, yeah. So, from your environment where you're coming from, maybe you can tell.

Interviewer: Oh, okay. So, uh what do you think uh might happen if people in your family or community found out that a girl was seeking sexual and reproductive health information or service?

Respondent: Pardon?

Interviewer: Uh, what do you think might happen if people in your family or community found out that a girl was seeking sexual and reproductive health information or services?

Respondent: W- what they may think? Okay. So, they may they may think that maybe this person has maybe been a victim a victim of sexual abuse or something, yeah. That's why maybe she or she or he is seeking information like, yeah, to get to know maybe of what happened, yeah. So, they may they may think that maybe he or she has gone through sexual abuse or something like that, yeah.

Interviewer: Mmm, uh so uh what about a family reaction? What- how will the family react?

Respondent: Okay, I think they'll be shocked, like, "Why are you asking about such information?" Yeah, so they may- they'll ask more. They'll ask more like, "What what happened?" Like, something s- like they'll want to know what what really made you to go seeking such information.

Interviewer: Mmm, okay. So, uh have you or someone you know ever experienced difficulties after seeking sexual and reproductive health information or services?

Respondent: Nope.

Interviewer: Okay. So, have you heard whether the experiences of girls in Kibera are similar to or different from those of girls in other parts of Nairobi or elsewhere in Kenya?

Respondent: Pardon?

Interviewer: Have you heard whether the experiences of girls in Kibera are similar to or different from those of girls in other parts of Nairobi or elsewhere in Kenya?

Respondent: No, I haven't heard whether they are similar or different.

Interviewer: Okay, so what changes do you think uh will make it easier for girls and young women in Kibera to access sexual and reproductive health information and services?

Respondent: Um-hum. Maybe campaigns, yeah. So, they'll be told like, yeah, the the goodness of having the information at right hand, yeah. Rather than assuming that everything is okay. So, through campaigns, yeah, and through talks, yeah. So, they be told the benefits of knowing your status and everything, yeah. I think that will help them to seek the facilities.

Interviewer: Okay, uh is there anything else about this topic that you think is important for us to understand that we have not discussed?

Respondent: Um-hum. Um-hum, at the moment, no. I don't have any.

Interviewer: Okay, thank you very much for taking the time to share your experience and perspective today. Your contribution is very valuable. And it will help us uh better understand the challenges young women face in sexual in accessing sexual and reproductive health information services.

H. Focus Group Discussion Summary Report (Raw Data)

This appendix reproduces the focus group discussion report as recorded by the research team, including the discussion summary and analysis notes for each question.

Perceptions of SRHR Services

Question 1: What do girls and young women in this community think about seeking SRHR services?

Discussion

Participants converged on SRHR being important in principle but underused in practice. Several described SRH services as necessary, confidential, and protective, framing access as essential for disease prevention and self-protection. At the same time, multiple participants admitted they personally only seek care when symptomatic rather than proactively, even while acknowledging that routine, preventive visits would likely be more beneficial since some conditions have no early symptoms.

A recurring point was that SRHR is widely misunderstood as being synonymous with sexual activity. One participant recounted having to clarify to a peer that seeking SRHR services doesn't imply someone is sexually active. The framework covers periods, contraception, pregnancy, and general reproductive health regardless of sexual history.

Negative provider interactions were also raised early: one participant described being misdiagnosed for a UTI as a sexually transmitted infection, and generally, participants felt providers reflexively assumed anything reproductive-health-related was sexual in origin.

Analysis

This suggests two distinct barriers operating simultaneously, a genuine knowledge/awareness gap (conflating SRHR with sex) and a practiced norm of reactive rather than preventive care-seeking, even among participants who articulate the value of prevention. This gap between stated belief and actual behavior is worth treating as its own theme rather than folding it into "stigma."

Question 2: What cultural barriers or community attitudes make it harder for girls to access these services?

Discussion

Several distinct mechanisms were raised:

FGM-practicing communities: complications from FGM (pain, bleeding, childbirth difficulty) were described as generating shame that discourages participants from seeking related care, compounding rather than only causing physical harm.

Family communication barriers: participants described being unable to discuss sexual/reproductive health with parents due to fear of judgment or being perceived as sexually active. Distrust of aunts and discomfort discussing periods or sex with fathers were both raised as normalized barriers.

Provider attitudes: multiple participants described feeling dismissed by gynecologists who characterized severe pain as "normal period pain," delaying diagnosis of more serious conditions for years in at least one case.

Religious/cultural framing: some participants noted that religious teachings discourage contraception specifically, creating tension between what clergy or community elders teach and what clinical guidance recommends.

Severe cases: one participant relayed a secondhand account (from a medical professional) of a patient from a community practicing genital sewing as FGM, and the complications that arose during childbirth, illustrating that "barrier" in some communities extends to serious physical harm, not just social discomfort.

One male participant noted that professionals' general absence from community spaces signals to boys/young men that these health topics aren't taken seriously, and that gender-segregated health education leaves boys without an equivalent peer support structure.

Analysis

Barriers span a spectrum from mild social discomfort (family conversations) to structural neglect (dismissive providers) to severe harm (FGM, forced closure practices). Grouping all of these under "cultural barriers" risks flattening very different severities and different points of intervention.

Family communication norms are addressable through education campaigns, while extreme practices require a different kind of intervention entirely.

Question 3: Where do girls get their SRH information and support, and is it trustworthy?

Discussion

Formal healthcare was not the dominant information source for most participants. Sources raised, in rough order of frequency:

AI tools (e.g., ChatGPT): one participant said she relies on AI more than she would confide in a sibling, but explicitly noted AI "can be manipulated" and isn't fully reliable, treating it as a supplementary rather than authoritative source.

Online communities and social media: described as valuable because they lower the barrier to learning compared to an in-person conversation, and because content on generational trauma/reproductive health was seen as actively shifting norms.

Peer networks: friends, including friends studying medicine, were cited as trusted sources.

Gynecologists: valued when trust was established, but participants noted that provider quality is inconsistent. While some dismiss concerns, others take them seriously.

Self-diagnosis/self-medication: several participants described routinely self-diagnosing based on online research or past experience, with explicit awareness of misdiagnosis risk (e.g., confusing a yeast infection with a UTI).

One participant disclosed she is personally developing an AI-based menstrual symptom triage tool, motivated by her own experience of providers dismissing her symptoms.

Analysis

The near-absence of formal healthcare as a first-line information source is a significant finding. It suggests distrust or inaccessibility of providers is pushing information-seeking toward informal, unverified channels (AI, peer networks, social media), which participants themselves flag as unreliable. This has direct implications for intervention design: peer- and tech-mediated channels may have more reach than clinic-based education campaigns, but need embedded accuracy safeguards.

Question 4: What happens socially, within families, or otherwise when it becomes known that a girl has sought SRHR services?

Discussion

Consequences were described as varying by the type of service sought:

Contraception (especially visible methods like implants): participants described being labeled "immoral" and subject to gossip.

HIV testing: described as carrying the highest level of stigma, with an automatic assumption of promiscuity.

Counseling: discussed as less overtly stigmatized than the above two, though still not neutral.

A gendered double standard also emerged: participants noted that women can obtain monthly contraceptives from a chemist with a vague pretext (e.g., citing heavy periods) without much suspicion, while men purchasing condoms are also judged, despite condoms serving a protective function widely endorsed in the group's own discussion. One participant noted this contradicted an initial assumption that only women face judgment around SRH purchases.

The "pads wrapped in newspaper" example recurred multiple times as shorthand for how menstruation-related purchases are treated as something to conceal even though menstruation is universal among women.

Analysis

The finding that stigma severity is service-specific (highest for HIV testing, lower for counseling) is a more precise and actionable finding than "stigma exists broadly": it suggests targeted destigmatization messaging could prioritize HIV testing and contraception specifically rather than treating all SRHR-seeking as equally stigmatized.

Question 5: Is the situation similar or different in other communities?

Discussion

Participants generally agreed the pattern (stigma, family silence, provider dismissiveness) is broadly shared across communities but is worse in specific contexts, pastoralist and some Muslim communities were named as facing compounded barriers, including practices like

FGM and early marriage layered on top of the general stigma described earlier in the discussion.

There was also a note of cautious optimism: participants pointed to social media platforms as spaces where mothers and younger women are increasingly discussing generational trauma and reproductive health openly, which several felt was slowly shifting norms, even if change is uneven across communities.

Analysis

This suggests the core stigma dynamics identified in this FGD (family silence, provider dismissiveness, service-specific judgment) are likely generalizable beyond this specific participant group, with severity as the key variable rather than the presence/absence of the barriers themselves. This is useful for the study's generalizability claims, provided it's caveated as self-reported perception of "other communities" rather than direct data.

Cross-Cutting Observations for Analysis

Behavior-belief gap: participants consistently endorse proactive/preventive SRHR use in the abstract but describe their own behavior as reactive/symptom-triggered, worth noting as distinct from stigma-driven avoidance.

Privacy assumption reversal: one participant stated she would feel less safe in a one-on-one session with an unfamiliar provider than in a group setting, which cuts against a common assumption that privacy is the primary condition for disclosure comfort.

Service-specific stigma gradient: HIV testing > contraception > counseling, in terms of stigma severity; a finding with direct implications for messaging design.

Informal/digital channels as primary information sources: formal healthcare providers were consistently described as secondary or a last resort relative to AI tools, peer networks, and social media.

Forward-looking participant input: the group itself proposed that future sessions shift from experience-sharing to solution-building, and specifically flagged the need to think beyond current participants (18+) toward younger girls, and male participant inclusion.